Ref: Ro J Neurol. 2025;24(Suppl)
DOI: 10.37897/RJN.2025.S.2

The 23rd National Congress of the Romanian Society of Neurology

POSTERS – ABSTRACTS

(in numerical order)

FUNDAMENTAL RESEARCH

P007. Insomnia and its association with fall risk among the elderly in western Romania: A retrospective cross-sectional study

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Introduction/objectives. Falls are common among the elderly and represent a major public health issue due to their significant social and economic impact and serious health consequences. The objective of this study was to investigate the associations between insomnia, comorbidities, multisite musculoskeletal pain, physical activity, and fall risk in older adults.

Materials and methods. This retrospective cross-sectional study included residents of elderly care homes in Timișoara, Romania. Participants aged 65 years and older were divided into two groups: those without fractures (Group I) and those with a history of fractures (Group II). Sleep quality was assessed using a single item from the Assessment of Quality of Life (AQoL) questionnaire on a 4-point scale. Fall risk was evaluated using the Falls Risk Assessment Tool (FRAT).

Results/Conclusions. A total of 140 participants were included, with a mean age of 78.4 ± 2.4 years (range: 65–98 years), of whom 55 were male (39%). Compared to participants without fractures, those with a history of fractures had significantly more comorbidities, higher fall risk scores, and more severe sleep disturbances.

Univariate logistic regression revealed significant associations between fracture occurrence and comorbidities, fall risk, and sleep disturbances ($p < 0.0001$). Multivariate regression identified four independent predictors of fractures: number of comorbidities ($p = 0.03$), fall risk score ($p = 0.006$), and sleep disturbances of type 3 ($p = 0.003$) and type 4 ($p = 0.001$). A FRAT score greater than 14 and more than two comorbidities were significantly associated with fracture risk. Strong positive correlations were also observed between the severity of sleep disturbances, fall risk scores, and fracture incidence.

Keywords: insomnia, falls, comorbidities, fractures, elderly

P008. Cognitive impairment is more prevalent among elderly nursing home residents with hearing loss in western Romania

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Introduction/objectives. The aim of this study was to investigate the correlation between the severity of neurocognitive disorders and hearing impairment in the elderly population.

Materials and methods. This population-based observational study included individuals aged 65 years and older. Participants were evaluated for cardiovascular risk factors, diabetes mellitus, history of stroke, alcohol use, and smoking. Hearing impairment was diagnosed by an audiologist through behavioral audiometric testing, either in response to patient concerns or as part of routine annual health assessments. Cognitive status was assessed using the Mini-Mental State Examination – 2nd Edition (MMSE-2). Based on MMSE-2 results, participants were categorized into two groups:

- Group I: those with cognitive impairment or severe neurocognitive disorder
- Group II: those without cognitive deficits

Results/conclusions. A total of 203 participants were included, with a mean age of 77 ± 7.5 years (range: 65–98), of whom 99 (48%) were male. Compared to Group II, participants in Group I had significantly higher rates of cardiovascular risk factors, stroke, diabetes, and hearing impairment.

Univariable logistic regression showed that cognitive impairment was significantly associated with cardiovascular disease, diabetes, and stroke ($p < 0.0001$). Multivariable regression analysis identified stroke ($p < 0.0001$), diabetes ($p = 0.0008$), cardiovascular disease ($p = 0.0004$), and hearing impairment ($p = 0.0011$) as independent predictors of cognitive impairment. Notably, hearing impairment was significantly associated with MMSE-2 scores ≤ 14.

These findings suggest that elderly individuals with hearing impairment—especially those with additional vascular risk factors—may be at increased risk of developing severe neurocognitive disorders.

Keywords: elderly, hearing impairment, cognitive impairment, MMSE-2, nursing home.

P033. Real-world reduction in oral its role in female fertility. A multiple sclerosis center analysis

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Introduction/objectives. This study aimed to evaluate oral corticosteroid (OCS) usage at 1 year following efgartigimod (EFG) initiation.

Materials and methods. Patients with generalized myasthenia gravis (gMG) using chronic OCS pre-EFG initiation were identified from a United States medical and pharmacy claims database (based on information licensed from IQVIA: Longitudinal Access and Adjudication Data [LAAD] for the period April 2016–December 2023, reflecting estimates of real-world activity [all rights reserved]). Mean (standard deviation [SD]) OCS average daily dose (ADD) was evaluated during the 3 months prior to, and at 6 and 12 months post-EFG initiation. To assess outcomes, de-identified Myasthenia Gravis Activities of Daily Living (MG-ADL) data collected in the „My VYVGART Path" patient support program were tokenized and integrated into the primary dataset..

Results/conclusions. 169 eligible adults (aged ≥ 18 years) initiated EFG by December 31, 2022, and continued EFG for at least 12 months were included in the analysis. At 6 and 12 months post-EFG, respectively, 31 (18%) and 45 (27%) patients had no OCS usage. Overall mean (SD) OCS ADD was significantly reduced at 6 months (13.2 [13.9] mg/day, $P < 0.001$), and at 12 months (10.2 [12.1] mg/day, $P < 0.001$) post-EFG initiation compared with baseline (17.2 [13.7] mg/day). Among a subset of 72 patients (43%) who had both pre- and post-EFG MG-ADL scores available, best follow-up mean (SD) MG-ADL was significantly improved (from 8.3 [3.7] to 3.4 [2.8], $P < 0.001$). The significant reduction of OCS usage observed at 6 months post-EFG initiation was retained at 12 months, while demonstrating MG-ADL response expected from EFG treatment.

P034. Functional movement disorders: diagnostic challenges and therapeutic perspectives

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Introduction/objectives. Functional movement disorders (FMD) represent a heterogeneous group of motor disorders with no clear organic cause and can be mistaken for neurological conditions such as Parkinson's disease or dystonia.

Materials and methods. A systematic review of recent literature and an analysis of clinical cases were conducted, including both published case studies and data from clinical practice. Current diagnostic methods and available therapeutic options were examined. Additionally, emerging research perspectives on innovative treatment technologies, such as deep brain stimulation and neurofeedback, were considered.

Results. FMDs are challenging to diagnose, as they can mimic symptoms of organic neurological disorders. Accurate diagnosis relies on the exclusion of other disorders and the use of a multidisciplinary arsenal of clinical and imaging tests. Therapeutic approaches, particularly cognitive-behavioral therapy and physiotherapy, have shown promising results in symptom improvement, although there is no standardized pharmacological treatment.

Conclusions. FMDs are complex disorders that require careful and differentiated diagnosis. A diagnostic algorithm is proposed to facilitate the accurate identification of these conditions through a structured clinical evaluation, the exclusion of organic causes, and the integration of a multidisciplinary approach, thereby contributing to improved diagnostic accuracy and personalized therapeutic strategies for patients.

Keywords: functional movement disorders, differential diagnosis, pharmacological treatment, behavioral therapy

P035. Retrospective longitudinal study regarding the effects of natalizumab and teriflunomide on brain atrophy in multiple sclerosis after five years of treatment

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Brain atrophy is a key marker of disease progression in multiple sclerosis (MS). The corpus callosum index (CCI) is a normalized measurement that reflects changes in brain volume in MS patients. While studies have evaluated the impact of both teriflunomide and natalizumab on brain atrophy, direct comparative data focusing on CCI between these two treatments are limited.

Materials and methods. We conducted a retrospective analysis of 10 patients, with five receiving natalizumab and five on teriflunomide as first-line disease-modifying therapies (DMT), each with a minimum of five years of treatment. We correlated the data with disease duration, age of the patients, and EDSS values. The CCI was calculated for each patient using MRIs obtained at baseline and after a minimum of five years of DMT.

Results. Preliminary findings suggest no significant difference in CCI values between the two groups ($p > 0.05$). The differences between EDSS from baseline to the most recent measurement were also not statistically significant.

Conclusions. In terms of cerebral atrophy and functional status, this study indicates no significant difference between patients receiving teriflunomide or natalizumab. These findings provide an open path for larger studies to further investigate the long-term effects of these treatments.

Keywords: multiple sclerosis, brain atrophy, natalizumab, teriflunomide, corpus callosum index

P036. Carpal tunnel syndrome in patients with type 2 diabetes mellitus: clinical and electrophysiological correlations

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Introduction. Carpal tunnel syndrome (CTS) is a common compressive mononeuropathy characterized by pain and paraesthesia in digits I–IV of the upper limb, typically worsening at night, as well as motor deficits in more advanced stages. Compared to the general population, patients with diabetes mellitus (DM) are considered at increased risk, with medical studies reporting a prevalence of 15–20%.

Materials and methods. We conducted a prospective clinical study involving 123 patients diagnosed with type 2 diabetes mellitus (DM2), who underwent electroneurography and were monitored for various laboratory and electrophysiological parameters.

Results. In this cohort, 48 patients were diagnosed with CTS. Among them, 11 patients (22.9%) had strictly unilateral left involvement, 15 patients (31.25%) had strictly unilateral right involvement, and 22 patients (45.83%) had bilateral involvement. Patients with CTS exhibited significantly higher HbA1c values (8.3% vs. 6.7%, $p = 0.008$). The presence of CTS correlated with a lower platelet-to-lymphocyte ratio (PLR) ($p = 0.03$). Additionally, CTS was associated with reduced amplitudes of sensory nerve action potentials (SNAPs) in the left sural nerve ($p = 0.000$) and right sural nerve ($p = 0.004$), and reduced compound muscle action potentials (CMAPs) in the left peroneal ($p = 0.019$), right peroneal ($p = 0.028$), and left tibial nerves ($p = 0.04$).

Conclusions. In patients with type 2 diabetes mellitus, CTS is associated with poor glycemic control, diabetic neuropathy, and subclinical chronic inflammation.

Keywords: carpal tunnel syndrome, type 2 diabetes, inflammation

P037. The inflammatory footprint of Parkinson's – linking systemic inflammation to clinical burden

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Introduction. The identification of relevant serological biomarkers for assessing clinical complexity in Parkinson's disease (PD) remains a key objective. While peripheral inflammatory markers have been implicated in PD pathogenesis, the association between hemoglobin levels and disease severity is still under debate.

Materials and methods. We conducted a study involving 20 patients diagnosed with PD, excluding those with systemic inflammatory disorders. The objective was to evaluate correlations between serological parameters, including peripheral inflammatory markers (C-reactive protein [CRP], erythrocyte sedimentation rate [ESR], fibrinogen), complete blood count elements (hemoglobin, lymphocyte-to-monocyte ratio [LMR], neutrophil-to-lymphocyte ratio [NLR], and platelet-to-lymphocyte ratio [PLR]), and clinical features. These included non-motor symptoms, disease severity, dopaminergic treatment-related complications, and disability scales.

Results. CRP levels showed significant positive correlations with several clinical parameters, including the number of daily levodopa doses, Hoehn & Yahr stage, and motor fluctuations (wearing-off, delayed-on, morning akinesia). CRP was negatively associated with the Instrumental Activities of Daily Living (IADL) score. ESR was positively associated with wearing-off phenomena, a range of non-motor symptoms, and the modified Rankin Scale (mRS), and negatively associated with the Barthel Index. Both NLR and PLR positively correlated with the severity of non-motor symptoms and autonomic dysfunction. Lower hemoglobin levels were associated with greater disease severity, more frequent motor fluctuations, and longer disease duration.

Conclusions. These findings suggest a potential role for systemic inflammatory biomarkers in evaluating Parkinson's disease severity and complexity. The results highlight the need for further studies on systemic

inflammation as a contributing factor in PD pathophysiology.

Keywords: Parkinson's disease, systemic inflammation, biomarkers, hemoglobin, non-motor symptoms, motor fluctuations

P038. Association of sleep disturbances with disease severity and disability in patients with Parkinson's disease: a cross-sectional study

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Background/objectives. Sleep disturbances such as insomnia, REM sleep behavior disorder (RBD), excessive daytime sleepiness (EDS), and sleep apnea episodes (SAEs) are frequent in patients with Parkinson's disease (PD). They aggravate motor and non-motor symptoms (e.g., cognition, depression, anxiety), impair quality of life, and contribute to disability.

Methods. We conducted a cross-sectional study enrolling patients with PD, aiming to determine the association between different sleep disorders and PD characteristics. The occurrence of RBD, EDS, and insomnia was revealed by direct questioning, whereas SAEs were determined using an ambulatory blood pressure monitoring device.

Results. We included 47 patients with a median age of 71 years, PD duration of 9 years, and MDS-UPDRS Part III score of 40. Patients with RBD were more prone to all types of levodopa-induced motor complications and cognitive impairment and scored higher on scales measuring overall non-motor burden and autonomic dysfunction. Patients with EDS had a higher degree of disability, longer off periods, and were more susceptible to delayed-on phenomena, freezing of gait episodes, urinary dysfunction, and fatigue. EDS was also associated with the use of dopaminergic agonists. Moreover, the presence of SAEs was positively associated with motor fluctuations such as wearing off and delayed on, psychiatric symptoms such as hallucinations and impulse control disorder, as well as urinary complaints.

Conclusions. Sleep disorders in PD patients are associated with levodopa-induced motor complications, neuropsychiatric symptoms, and autonomic dysfunction. Whether these disorders represent an epiphenomenon or contribute causally to PD severity remains a topic of ongoing debate.

CASE PRESENTATION

P047. Carotid artery dissection presenting as isolated unilateral headache with cranial autonomic features

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Introduction. Unilateral headache can conceal a potentially life-threatening condition. Dissection of the carotid artery may present solely with headache, without any focal neurological signs. Therefore, a meticulous differential diagnosis is essential.

Methods. We examined two cases of internal carotid artery dissection where unilateral headache was the main clinical manifestation, with no accompanying cerebrovascular symptoms.

Results. We report two cases of new-onset unilateral headache secondary to spontaneous internal carotid artery dissection in two male patients (aged 60 and 39 years). Neurological examination in both cases revealed no focal deficits. However, ipsilateral eyelid edema and mild ptosis were noted. Magnetic resonance angiography (MRA) of the head and neck confirmed the diagnosis of internal carotid artery dissection and excluded other causes. Both patients were treated conservatively with aspirin and rivaroxaban, without requiring endovascular or surgical intervention.

Conclusion. A high index of suspicion for carotid artery dissection should be maintained in patients presenting with isolated unilateral headache accompanied by cranial autonomic features.

P048. Therapeutic solution for a patient with cerebral hemorrhage, infraclinical atrial fibrillation, and contraindication for anticoagulation – case report

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Clinical case. A 76-year-old female, diagnosed with renal polycystosis and end-stage renal disease, started hemodialysis in September 2024. She presented with paroxysmal atrial fibrillation and isolated Broca's aphasia. A contrast-enhanced cerebral CT scan revealed a right ischemic sylvian stroke. Cardiac and carotid echography and 24-hour ECG Holter were initially normal. Anticoagulation therapy was initiated but was complicated by a cerebral hemorrhage, which developed on a background of cerebral amyloid angiopathy (confirmed by MRI). Broca's aphasia progressively resolved.

After one month, the patient presented with recurrent confusion and aphasia. Repeated cerebral MRI showed multiple new ischemic lesions. Cardiac and carotid investigations remained normal. Infraclinical atrial fibrillation was suspected despite a normal 24-hour Holter ECG. Due to the hemorrhagic stroke and confirmed amyloid angiopathy, anticoagulation was contraindicated and discontinued. The patient underwent percutaneous closure of the left atrial appendage in the cardiac catheterization lab.

Three months post-intervention, the patient showed good clinical progress, with no further need for anticoagulation and no neurological deficit.

Conclusions. Percutaneous closure of the left atrial appendage may represent a viable solution for patients with atrial fibrillation who have a contraindication to anticoagulation.

Keywords: hemorrhagic stroke, amyloid angiopathy, left atrial appendage

P049. CANVAS on a canvas – three traits of a brush

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Introduction. CANVAS syndrome (cerebellar ataxia, neuropathy, vestibular areflexia syndrome) is a rare form of adult-onset hereditary cerebellar ataxia, accompanied by sensory neuropathy and vestibular areflexia.

Methods. We report the case of a 50-year-old female patient admitted to our clinic for gait impairment, imbalance, limb incoordination, dysarthria, numbness of the lower limbs, and chronic dry cough. The patient's symptoms had an insidious onset and progressed slowly over approximately 10 years.

Results. The patient underwent comprehensive investigations to exclude other common etiologies of ataxia (including metabolic, toxic, autoimmune causes, and genetic testing for common hereditary ataxias). Nerve conduction studies revealed severe sensory axonal polyneuropathy with absent sensory responses, and vestibular testing demonstrated bilateral vestibular hyporeflexia. Brain MRI revealed significant cerebral and cerebellar atrophy, predominantly involving the vermis, disproportionate to the patient's age. Ataxia rating scales were employed to assess the degree of disability.

Given the disease history, thorough workup excluding alternative diagnoses, and the presence of the classic diagnostic triad, the patient was referred for genetic testing to detect expanded AAGGG repeats in intron 2 of the RFC1 gene.

Conclusions. CANVAS should be considered in patients with the diagnostic triad of cerebellar ataxia, polyneuropathy, and vestibular areflexia, particularly

when accompanied by chronic cough, and after ruling out more common causes of hereditary ataxia.

Keywords: CANVAS, sensory polyneuropathy, vestibular areflexia, hereditary cerebellar ataxia, chronic dry cough

P050. A diagnostic dilemma: from HSV encephalitis to tumor diagnosis

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Introduction. Viral encephalitis is a life-threatening condition that often debuts with fever, seizures, and neurological deficits. Personality changes and cognitive impairment have also been described. Herpes simplex virus (HSV) encephalitis is common, but its diagnosis can be challenging, often overlapping with other neurological conditions.

This case illustrates a patient initially suspected of having HSV encephalitis, later proven to have a brain tumor.

Case presentation. A 74-year-old female with no significant medical history presented with transient visual disturbances, memory impairment, speech disorder, and a seizure-like episode. The symptoms began two weeks prior to the initial clinical visit. Neurological examination revealed dizziness when standing and lateral deviation. Initial imaging, including brain MRI, emphasized abnormalities in the left temporal cortex with occipital extension, raising suspicion for HSV encephalitis. CSF analysis was not consistent with viral encephalitis.

Diagnostic workup. Repeated contrast-enhanced MRI with spectroscopy revealed an evolving left temporal lobe mass. MR spectroscopy showed choline/creatinine elevation with reduced N-acetylaspartate (NAA), suggesting a glioma. Further investigations, including a biopsy, confirmed the diagnosis of a primary brain tumor.

Final diagnosis. The patient was ultimately diagnosed with a malignant brain tumor, rather than HSV encephalitis.

Discussion. This case emphasizes the importance of maintaining a broad differential diagnosis when faced with patients with focal neurological deficits and suggestive brain imaging findings. While HSV is the leading cause of viral encephalitis, brain tumors should also be considered. Early diagnosis of brain tumors is critical for appropriate treatment and improving patient prognosis.

Keywords: HSV encephalitis, glioma, diagnostic dilemma, MRI imaging, CSF analysis, seizures

P051. Fibromuscular dysplasia – a rare cause of acute ischemic stroke: a case report

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Introduction. Fibromuscular dysplasia (FMD) is a relatively uncommon, non-atherosclerotic, non-inflammatory vascular condition that primarily affects medium-sized arteries. The most commonly involved vessels are the renal, carotid, and vertebral arteries. Altered blood flow promotes stasis and thrombus formation, which can embolize distally as intra-arterial thromboemboli, leading to ischemic stroke. Bilateral internal carotid artery involvement is present in approximately 80% of cases.

Case presentation. We present the case of a 42-year-old female smoker with a history of pregnancy loss, hypertension, hereditary thrombophilia (treated with anticoagulants), and breast cancer (treated with surgery and radiation therapy). The patient had multiple neurological admissions for symptoms consistent with bilateral carotid artery stroke, each time making a full recovery. She presented in the outpatient setting complaining of headache, cervical pain, and tinnitus. Clinical and neurological examinations were unremarkable. Cerebral MRI revealed chronic bilateral ischemic strokes in the superficial middle cerebral artery territories, and MR angiography revealed severe stenosis (approximately 80%) in the cervical segment of the right internal carotid artery and in the petrous segment of the left internal carotid artery. A characteristic "string of beads" appearance was observed in the cervical segments of both vertebral arteries, suggestive of fibromuscular dysplasia. Antiplatelet therapy was added to anticoagulants, resulting in a favorable neurological outcome, with no new acute events.

Following an episode of abdominal angina, an abdominal CT angiography revealed normal renal arteries and abdominal aorta, but occlusion of the celiac trunk with distal patency, proximal occlusion of the superior mesenteric artery with distal flow, and hyper-

plasia of the inferior mesenteric artery, along with a rich network of colic and enteral anastomoses.

Conclusion. FMD is an uncommon vascular condition that represents an important etiology of ischemic stroke, particularly in young women. It should be included in the differential diagnosis of ischemic stroke, especially when bilateral cerebral lesions are present alongside symptoms such as headache, neck pain, and tinnitus. The coexistence of FMD and hereditary thrombophilia likely contributed to the patient's cerebrovascular events through both structural and prothrombotic mechanisms.

Keywords: fibromuscular dysplasia, string of beads, cerebral angiography, stroke

P052. Ischemic stroke in the context of central nervous system vasculitis at a young age

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Introduction. Central nervous system (CNS) vasculitis represents a heterogeneous group of inflammatory diseases affecting the blood vessel walls of the brain, spinal cord, and meninges. Clinically, it commonly presents with headache, cognitive impairment, and focal neurological deficits (e.g., motor weakness, aphasia, visual disturbances). We present the case of a 31-year-old patient with sudden-onset left-sided muscle weakness, in whom two acute ischemic strokes and cerebral plurivascular changes were detected, suggestive of Takayasu vasculitis type I.

Case presentation. A 31-year-old patient with multiple comorbidities, including psoriasis vulgaris, psoriatic arthropathy, and type 1 diabetes mellitus, presented to the emergency department for sudden-onset left-sided weakness. Angio-CT revealed two acute ischemic lesions and diffuse parietal changes in cerebral vessels, raising the suspicion of vasculitis. After multidisciplinary evaluation, a diagnosis of Takayasu vasculitis type I was established. Specialized treatment was initiated, resulting in improvement of both neurological and rheumatological symptoms.

Conclusions. Although CNS vasculitis is a rare cause of stroke, it should be promptly recognized, particularly in young patients presenting with ischemic events. Early diagnosis and targeted therapy can significantly improve patient outcomes.

Keywords: CNS vasculitis, ischemic stroke, young age

P053. Intracranial hemorrhage in postpartum: a rare complication of epidural anesthesia

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Intracranial hemorrhage rarely occurs in the peripartum period. The most frequent causes of this rare and potentially life-threatening complication include eclampsia, rupture of arteriovenous malformations or arterial aneurysms, intracranial venous thrombosis, postpartum reversible cerebral vasoconstriction syndrome, coagulopathies, and complications of epidural or spinal anesthesia.

We report a rare case of simultaneous subdural and subarachnoid hemorrhage after epidural anesthesia occurring in the postpartum period.

A 32-year-old secondiparous woman, known to have bronchial asthma, experienced the worst headache of her life within hours after cesarean section and did not respond to analgesics. The C-section was initially performed under epidural anesthesia, but due to intraoperative complications, the anesthesia was converted to general anesthesia. On the sixth postpartum day, the patient developed a tonic-clonic seizure. She exhibited no signs of eclampsia before or after delivery and denied any head trauma.

CT angiography, performed after the seizure, revealed a subacute subdural hematoma in the right frontoparietal area and subarachnoid hemorrhage, with no signs of arterial aneurysm, arteriovenous malformation, or cerebral venous thrombosis. MRI angiography, performed one month later, excluded the possibility of a ruptured arterial aneurysm.

An intense headache that begins shortly after epidural anesthesia can be a warning sign of intracranial hemorrhage and should prompt urgent investigation.

Keywords: epidural anesthesia, subdural hemorrhage, subarachnoid hemorrhage, postpartum

P054. A late-onset congenital myasthenic syndrome due to heterozygous RAPSN mutation

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Background. Congenital myasthenic syndrome type 11 is a genetic disorder caused by mutations in the RAPSN gene, which encodes rapsyn, a protein essential for anchoring acetylcholine receptors to the postsynaptic membrane.

Case presentation. We report the case of a 46-year-old man with slowly progressive muscle weakness beginning at the age of 26. The motor deficit gradually extended to all limbs by age 41. His medical history included bilateral congenital hip dislocation, for which he had undergone multiple orthopedic surgeries.

Prior to admission to our hospital, electromyography (EMG) revealed a myogenic pattern. The patient tested negative for Pompe disease, and a muscle biopsy yielded inconclusive results. Based on clinical and EMG findings, a diagnosis of limb-girdle muscular dystrophy was initially suspected.

Upon hospital admission, he presented with symmetrical, predominantly proximal motor weakness in all limbs. Symptoms worsened after physical activity and in the evening. Repetitive nerve stimulation of the trapezius muscle showed approximately 50% decrement. Whole exome sequencing (WES) subsequently revealed a heterozygous RAPSN mutation, confirming the diagnosis of congenital myasthenic syndrome type 11. The patient showed clinical improvement under treatment with pyridostigmine.

Conclusions. Although congenital myasthenic syndromes typically present in early childhood, rare cases may debut in adulthood. The late onset and diagnostic delay due to confounding orthopedic comorbidities are key features of this case. This highlights the importance of WES in establishing a correct diagnosis when clinical presentation is atypical.

Keywords: congenital myasthenic syndrome, rapsyn, RAPSN mutation, whole exome sequencing

P055. Ataxic tetraparesis – an atypical manifestation of limbic encephalitis with positive onconeuronal antibodies

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Introduction. Limbic encephalitis is an autoimmune encephalitis characterized by inflammation of the limbic structures of the brain, including the cingulate cortex, frontobasal cortex, hippocampus, and medial temporal lobe. It may have a paraneoplastic or non-paraneoplastic etiology. The primary pathophysiological mechanism is autoimmune, involving antibodies directed against intracellular antigens or proteins expressed on the cell surface.

Presentation. We report the case of a 67-year-old patient with subacute onset of decreased muscle strength in all limbs, gait disturbance, and memory impairment. Neurological examination revealed unsteady gait with bilateral support, ataxic tetraparesis, intention and resting tremor in the right upper limb, paresthesias in all limbs and perioral region, apallesthesia in the distal lower limbs, dysmetria on finger-to-nose and heel-to-knee tests (right side), mild cognitive impairment, sleep disturbances, and myoclonus.

Brain MRI revealed T2 and T2-FLAIR hyperintensities in the hippocampus and cerebellar tonsils. Electroencephalogram (EEG) showed bilateral fronto-temporo-parietal epileptiform activity. Laboratory tests were positive for IgG onconeuronal antibodies: anti-CV2, anti-Ri, anti-Hu, and anti-SOX1+. The patient underwent five sessions of plasmapheresis, resulting in slight clinical improvement.

Conclusions. Despite partial improvement with plasmapheresis, the prognosis remains guarded, particularly because extensive investigations failed to identify an underlying neoplasm.

Keywords: encephalitis, limbic structures, onconeuronal antigens, neoplasm

P056. Atypical case of Miller Fisher syndrome with positive anti-GAD antibodies and post-immunoglobulin therapy complications

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Introduction. Miller Fisher syndrome (MFS) is a variant of Guillain-Barré syndrome, characterized by the triad of ophthalmoplegia, ataxia, and areflexia. It is classically associated with anti-ganglioside (GQ1b) antibodies, though a significant proportion (>10%) of cases are seronegative. We report an atypical case of MFS, negative for anti-ganglioside antibodies but positive for anti-GAD antibodies, suggesting a potential alternative pathogenic mechanism. The disease course was complicated by thrombocytopenia and hepatocellular injury following intravenous immunoglobulin (IVIG) therapy.

Case presentation. A 36-year-old patient with no significant medical history was admitted for diplopia, bilateral external ophthalmoplegia, ataxia, and areflexia. Serological tests revealed positive anti-glutamic acid decarboxylase (anti-GAD) antibodies, while anti-ganglioside antibodies, including anti-GQ1b, were negative. Cerebrospinal fluid analysis revealed albuminocytologic dissociation, and nerve conduction studies showed mild temporal dispersion in the left ulnar nerve.

The patient received intravenous methylprednisolone (total 4 g), followed by IVIG (total 40 g). However, therapy was discontinued prematurely due to elevated liver enzymes and thrombocytopenia. Despite complications, the patient experienced a gradual and complete recovery over six months.

Conclusions. This case broadens the clinical spectrum of Miller Fisher syndrome, suggesting that anti-GAD antibodies may contribute to pathogenesis in seronegative patients. It also underscores the importance of monitoring for rare hematologic and hepatic complications of IVIG therapy.

Keywords: Miller Fisher syndrome, anti-GAD antibodies, IVIG complications, ophthalmoplegia, albuminocytologic dissociation

P057. Gestational choriocarcinoma – neurological complications: a complex case with a multidisciplinary approach

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Introduction. Gestational choriocarcinoma is a rare and aggressive tumor arising from abnormal trophoblastic proliferation. We present a case with significant neurological involvement, emphasizing the importance of early diagnosis and a multidisciplinary approach.

Case presentation. A 33-year-old woman, three months postpartum following an uneventful term delivery, presented with fatigue and weakness of the left upper limb progressing over three days, followed by altered consciousness on the day of admission.

Management and outcomes. Neurologically, the patient was stuporous, with divergent strabismus of the left eye, tetraplegia, and inadequate bilateral flexion to nociceptive stimuli.

Paraclinical investigations: Brain CT and thoraco-abdomino-pelvic CT revealed a heterogeneous, predominantly hyperdense right frontal intracerebral mass with perilesional edema, a filling defect in the left atrium suggesting an intra-atrial tumor, and multiple infarcts (splenic, renal, pulmonary).

Further investigations (brain MRI and cerebral angio-MRI) showed cortico-subcortical emboli in the cerebellum, temporal, frontal, and bilateral parietal lobes, and an expansive right frontal lesion with peripheral contrast enhancement.

The patient was referred to cardiac surgery, where a mini-thoracotomy was performed, and the intra-atrial tumor was resected. Histopathology confirmed a necrotic thrombus with viable choriocarcinoma cells.

Systemic induction chemotherapy with cisplatin and etoposide was initiated, along with stereotactic body radiotherapy (SBRT) to cerebral lesions. The patient showed clinical improvement and radiological regression of tumor size.

Conclusions. This case highlights the complexity of gestational choriocarcinoma with neurological involvement. Early identification and multidisciplinary coordination are essential for favorable outcomes in such rare and life-threatening conditions.

Keywords: gestational choriocarcinoma, cerebral emboli, intra-atrial thrombus

P058. Reversible cerebral vasoconstriction syndrome – a case report

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Introduction. Reversible cerebral vasoconstriction syndrome (RCVS) includes conditions characterized by multifocal transient narrowing of cerebral arteries, often presenting with "thunderclap" headaches, which may sometimes be accompanied by other neurological signs (motor deficits, aphasia, ataxia).

Materials and methods. We present the case of a 40-year-old male smoker with no significant medical history who initially presented to the emergency department with right facial-brachial paresthesia and transient speech disturbances following intense physical activity. Neurological examination during hospitalization showed no focal deficits, and initial brain imaging (non-contrast CT, angio-CT, and MRI) revealed no abnormalities.

Two days later, the patient developed a sudden "thunderclap" headache unresponsive to analgesics. Doppler ultrasounds indicated high-velocity flow in the middle cerebral arteries (200 cm/s left, 150 cm/s right). A four-vessel cerebral angiography revealed left middle artery vasospasm, which resolved with intra-arterial nimodipine.

Discussion and conclusion. Following clinical assessments and imaging findings, treatment with nimodipine was initiated for 21 days; however, no significant therapeutic effect was noted. Subsequently, low-dose amlodipine was administered despite its absence in current clinical guidelines. Remarkably, the patient experienced substantial headache relief, with complete remission within 1 to 2 days of starting amlodipine. Follow-up Doppler ultrasounds at one, two, and three months demonstrated decreased flow velocities in the middle cerebral arteries (135 cm/s left, 120 cm/s right). Although the reduction in flow velocities was gradual, the rapid alleviation of headache pain highlights the potential effectiveness of amlodipine in treating RCVS.

Keywords: reversible cerebral vasoconstriction, thunderclap headache, amlodipine

P059. Progressive brainstem syndrome associated with multiple myeloma – a case report

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Introduction. Paraneoplastic neurological syndromes can involve both the central and peripheral nervous systems. They are a heterogeneous group of immune-mediated diseases associated with cancer, caused by a cross-reactive immune response between tumor antigens and neuronal antigens.

Objective. We present the case of a man with signs of progressive brainstem involvement and sensorimotor polyneuropathy, who was later diagnosed with multiple myeloma.

Case presentation. A 65-year-old man began experiencing symptoms in 2023, including epileptic seizures and oculomotor disturbances, with progressive evolution toward complete horizontal gaze palsy. Over time, he also developed cranial nerve palsies in the bulbopontine region. Native and contrast-enhanced brain MRI did not reveal significant pathological changes, except for the gradual appearance of hemosiderin deposits, predominantly in the brainstem. Electromyographic examination showed a progressive sensorimotor polyneuropathy, in the absence of any apparent aggravating factors. Oncological screening revealed elevated IgG levels without evidence of a solid tumor, along with the presence of anti-recoverin antibodies in serum.

Given the clinical and imaging findings, an extensive evaluation was performed (including CSF analysis, screening for autoimmune encephalitis and amyloidosis, autoimmune disease panel, and ophthalmoscopy).

Conclusions. This case suggests that brainstem involvement and polyneuropathy may represent phenotypes of nervous system impairment through an immune-mediated mechanism possibly associated with multiple myeloma.

P060. A strange stroke mimic: human herpesvirus 6 encephalitis (case report)

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Background. Human herpesvirus 6 (HHV-6) is a virus known for causing the highly contagious infection roseola infantum and encephalitis in the pediatric population, but less commonly in adult patients, generally affecting immunocompromised hosts.

We present a rare case of HHV-6 encephalitis in a previously immunocompetent male, with only a few apparent immune risk factors.

Case presentation. A 58-year-old male with a history of uncontrolled type 2 diabetes (HbA1c = 8.9%) and common cardiovascular risk factors presented as a stroke code for acute disorientation, agitation, and left hemineglect.

In the emergency department, he was highly aggressive, overactive, and uncooperative to history and examination, necessitating mild sedation repeatedly.

Upon arrival, he was afebrile, with an unremarkable biological inflammatory syndrome and slightly elevated blood glucose, showing no signs of meningeal inflammation, and a non-contrast brain CT was unremarkable.

Between the episodes of hyperactive delirium, we observed elements of aphasia and left hemineglect, prompting the initiation of IV thrombolysis.

However, symptoms persisted after admission, raising suspicion of meningoencephalitis, later confirmed by lumbar puncture (positive PCR for HHV-6). Repeated brain imaging through CT scan at 24 hours excluded vascular lesions.

Antiviral treatment with aciclovir was initiated and subsequently switched to ganciclovir after lumbar puncture. Later, MRI of the brain was performed and showed typical acute enhancement of the right mesial temporal lobe.

Conclusion. This case highlights the variety of signs and symptoms of acute encephalitis presenting as a stroke mimic, treated with IV tPA before switching to antiviral agents, to which the patient responded favorably and was discharged with no sequelae.

P061. Encephalomyelopolyradiculoneuritis – an ongoing diagnostic labyrinth

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Pathological conditions affecting both central and peripheral nervous systems simultaneously pose a diagnostic and therapeutic challenge.

We present the case of a 38-year-old, previously healthy male who presented in our clinic 40 days after the onset of slowly progressive ascending paresthesia and motor dysfunction, preceded two weeks earlier by a respiratory infection. Methylprednisolone treatment was administered in another clinic, acutely worsening the deficit. Lumbar puncture prior to treatment was normal, with no oligoclonal bands. Anti-MOG and AQP4 antibodies were negative.

At admission, the patient was febrile, tetraparetic and paraplegic, with brisk lower limb reflexes, bilateral Babinski sign, thermoalgesic hypoesthesia with a T10 sensory level, lower limb myoarthrokinetic hypoesthesia, urinary retention, constipation, and bradycardia. Another lumbar puncture was performed, showing 41 mononuclear cells and negative PCR multiplex. Intravenous immunoglobulin was administered, along with dexamethasone and empiric antibiotherapy.

Lumbar puncture three days after treatment retrieved normal results. The electroneuromyographies, performed 7 days apart, showed motor radiculitis initially, followed by axonal motor neuritis. Full spine and cerebral MRI showed T2/FLAIR diffuse, varying, poorly defined hyperintensities within the cervicomedullary junction. Complete infection panel, including *Borrelia*, and malignancy screening were negative. A diagnosis of encephalomyelopolyradiculoneuritis was established. The evolution was slightly favorable, with remission of dysautonomic features and upper limb motor deficit, and minor improvement of paraplegia. Recurrent fever episodes persisted, with negative infectious workup, but remitted before discharge.

Immune-mediated neurological diseases can be difficult to diagnose, as numerous differential diagnoses need to be excluded, especially when dealing with a complex clinical picture. Starting empiric treatment is crucial in determining patient outcome.

Keywords: polyradiculoneuritis, encephalomyelitis

P062. Recurrent transient ischemic attacks revealing a carotid web in a young patient without vascular risk factors – a case report

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Background. Carotid web (CW), an intimal variant of fibromuscular dysplasia, is recognized as a cause of recurrent transient ischemic attacks (TIAs) and strokes, particularly in young patients without vascular risk factors.

Methods. A 39-year-old male presented with three stereotypical episodes of transient expressive aphasia. The initial episode included mild right-hand weakness and right central facial paresis, lasting 20 minutes. Subsequent episodes were shorter (10–15 minutes). Neurovascular imaging revealed a 1.3 mm triangular endoluminal protrusion at the posterior wall of the left carotid bulb, consistent with carotid web. No ischemic lesions were identified on CT, and intracranial vessels were unremarkable. Transcranial Doppler with bubble test excluded a patent foramen ovale. Routine EEG, laboratory findings, and autoimmune markers were unremarkable.

Results. CW-associated TIAs are likely secondary to turbulent flow and microemboli originating from the web structure. Dual antiplatelet therapy and statin therapy were initiated, with no other events at follow-up. Given the patient's recurrent episodes, endovascular stenting may be considered, as evidence-based guidelines are lacking.

Conclusion. This case highlights the importance of recognizing carotid web as an underdiagnosed cause of recurrent TIAs in young patients without vascular risk factors, as the diagnosis was missed during his first hospital admission. Early diagnosis can truly make a difference in preventing long-term complications.

Keywords: carotid web, transient ischemic attack, fibromuscular dysplasia, vascular risk factors

P063. Miyoshi myopathy: from the first symptoms to the genetic diagnosis

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Introduction. Miyoshi myopathy is a rare form of distal muscular dystrophy caused by mutations in the DYSF gene, which codes for the dysferlin protein. We present the case of a 24-year-old patient, who presented with decreased muscle strength in the lower limbs, difficulty walking and climbing stairs, with slowly progressive evolution over time, along with muscle atrophy in the calf muscles.

Methods. We analyzed the medical history of a 24-year-old patient, who presented with decreased muscle strength and significant amyotrophy in the calf muscles.

Results. The clinical examination highlighted the fact that the patient could not maintain standing and walking on tiptoes/heels, a paraparetic motor deficit accompanied by diminished osteotendinous reflexes, muscle hypotonia in the lower limbs, and significant amyotrophies, more pronounced in the calves. In the upper part of the body, no significant changes were noted. Histopathological examination revealed features characteristic of myocytolysis and chronic myositis. The electromyographic examination revealed a myopathy with a distal > proximal damage pattern in the lower limbs, with features of denervation and mild damage in the upper limbs, paraspinal. Biological explorations revealed an important rhabdomyolysis syndrome.

Finally, the definitive diagnosis was made through molecular analysis using a panel of 45 genes involved in the etiology of muscular dystrophies, which confirmed the presence of a dysferlinopathy – the Miyoshi form.

Conclusion. Miyoshi myopathy is a pathology that can often be underdiagnosed due to its insidious onset. Genetic testing plays a crucial role in confirming the diagnosis, and increasing awareness of early symptoms can lead to earlier diagnosis.

Keywords: miyoshi myopathy, dysferlinopathies

P064. A rare case of Moyamoya disease involving unilateral intracranial carotid artery stenosis

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Moyamoya is a rare cerebrovascular disorder of unknown cause, characterized by gradual narrowing of major intracranial arteries, accompanied by the development of prominent small-vessel collaterals. As a progressive condition, Moyamoya can lead to ischemic stroke or intracranial hemorrhage in both children and adults.

We present the case of a 42-year-old female patient with a known interatrial septal aneurysm and a history of a right capsular ischemic stroke, which occurred two years prior to her admission at our clinic, manifested by left-sided hemiparesis. A cerebral MRI revealed narrowing of the right internal carotid artery in its terminal segment. The imaging workup was extended with cerebral angiography, which demonstrated progressive narrowing of the right internal carotid artery, distal to the ophthalmic segment, extending to the clinoid segment, along with significant hypoplasia of the anterior cerebral artery in the A1 segment.

Further etiological work-up included serologic testing to assess for typical vascular risk factors, systemic vasculitis, or other potential pathologies that could lead to cerebral arterial stenosis, as well as cerebral Doppler ultrasound with microbubble injection to investigate the presence of a right-to-left shunt. However, all of these investigations were negative.

Given the angiographic findings suggestive of Moyamoya disease and the absence of other potential pathologies, we established the diagnosis of possible unilateral Moyamoya syndrome, stage I according to the Suzuki classification.

This rare case emphasizes the importance of investigating the etiology of an acute stroke, particularly in young patients, to guide subsequent therapeutic strategies and improve the overall outcome and prognosis for the patient.

Keywords: Moyamoya, cerebral angiography, unilateral arterial stenosis

P065. Amyotrophic lateral sclerosis – a diagnostic challenge

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Amyotrophic lateral sclerosis (ALS) is a progressive neurodegenerative disease, characterized by the impairment of central and peripheral motor neurons, leading to muscle weakness, spasticity, atrophy, and ultimately respiratory failure.

This case analyzes the evolution of a 51-year-old female patient initially diagnosed with primary lateral sclerosis (PLS), Mills variant, and later with ALS.

The disease onset occurred four years ago, with the patient initially presenting only upper motor neuron (UMN) signs—muscle weakness in the left hand, followed by involvement of the left lower limb three years later, progressing to spasticity and the onset of dysarthria one year ago. Imaging investigations, electrophysiological studies (electroneuromyography – ENMG, motor evoked potentials – MEP), and serological tests were performed to exclude other conditions, leading to the initial diagnosis of PLS, Mills variant (slowly progressive spastic hemiparesis). Upon reevaluation (after five months), the emergence of lower motor neuron (LMN) signs and worsening UMN signs were observed. According to the Gold Coast criteria (2019), a definitive ALS diagnosis was established, despite the absence of suggestive ENMG findings.

The conclusion highlights an initially slow progression (characterized by UMN signs for four years), followed by rapid deterioration (five months) with the appearance of LMN signs in a 51-year-old female patient (ALS is more commonly seen in males). Despite current therapeutic options (riluzole, clinical trials with rasagiline), the prognosis remains poor. This case underscores the importance of a careful differential diagnosis and continuous monitoring to adapt therapeutic strategies and improve patient quality of life.

P066. From ischemic stroke to dural arteriovenous fistula – a diagnostic challenge

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Dural arteriovenous fistulas represent direct arteriovenous connections without interposition of a capillary network, with high hemorrhagic and epileptogenic risk.

We present the case of an 86-year-old female, transferred from a regional clinic for speech impairment and right-sided hemiparesis, with the diagnosis of acute ischemic stroke in the territory of the left middle cerebral artery. Medical records mention a recent admission to the intensive care unit for a reversible episode of loss of consciousness, requiring respiratory support.

Brain computed tomography (CT) described a left temporoparietal ischemic lesion. CT angiography revealed unstable atheromatous plaques in the aortic arch and thrombosis of the left sigmoid sinus. Magnetic resonance imaging (MRI) further refined the differential diagnosis, supporting the diagnosis of a hemorrhagic venous infarction. The suspicion of a left temporo-occipital arteriovenous malformation was raised. Digital subtraction angiography (DSA) confirmed the presence of a left sigmoid dural arteriovenous fistula (AVF) with blood supply from the left occipital artery, with perimedullary drainage into the veins of the brainstem, Cognard type IIa+b.

In conclusion, we are dealing with a case of a patient diagnosed with a dural AVF potentially due to cerebral venous thrombosis, with intraparenchymal hemorrhage and possible epileptogenic phenomena (the episode of loss of consciousness can be interpreted as a non-convulsive epileptic status, in the absence of conclusive evidence). Therefore, this case demonstrates that the diagnosis could have been initially overlooked, mimicking an ischemic stroke, which carries a very high mortality risk.

P067. An unexpected cause of delayed cerebral hemorrhage in a patient with DBS

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Introduction. Deep brain stimulation (DBS) is indicated for advanced Parkinson's disease, as well as primary dystonia and other similar conditions. Potential adverse effects include local hemorrhages (usually within the first 48 hours peri-procedurally, rarely delayed up to 3 months post-implantation).

Materials and methods. We present the case of a 54-year-old male patient, obese and hypertensive, diagnosed with generalized primary dystonia, treated with DBS at the GPi level since 2017, with associated oral VMAT2 inhibitors, who was hospitalized due to sudden onset of right-sided hemiplegia and aphasia.

Results. Cerebral computed tomography (CT) revealed a left frontal intraparenchymal hematoma with significant early perilesional edema, disproportionate to the hematoma's volume. Dynamic brain magnetic resonance imaging (MRI) with contrast agent and vascular sequences posed differential diagnostic challenges with venous infarction, arteriovenous malformations, hemorrhagic-transformed arterial infarction, ultimately suggesting imaging features consistent with bleeding within a malignant tumor.

Conclusions. Late cerebral hemorrhage following DBS has an incidence reported at up to 3.7%, with unclear causes, a proposed mechanism being microvenular damage caused by electrode migration. Exceptional cases occurring two years post-procedure have been reported. In the presented case, the hemorrhage was intratumoral, with proximal electrode displacement due to mass effect. Although there appears to be no direct link between the malignant tumor and DBS, the literature does speculate on possible connections.

Keywords: hemorrhage, tumor, edema, DBS, dystonia

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P068. Long-term management of a multiple sclerosis patient: adaptive treatment strategies and balancing risks

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Introduction. Multiple sclerosis (MS) is a chronic autoimmune disorder that demands individualized treatment strategies due to its unpredictable progression and potential for significant disability. This case presents the 14-year journey of a female patient with relapsing-remitting MS, highlighting how her treatment evolved over time in response to disease progression and adverse effects, with a particular focus on balancing efficacy and safety.

Methods. Diagnosed at age 30, the patient initially presented with diplopia, facial asymmetry, and sensory disturbances, with MRI confirming multiple lesions consistent with MS. Her treatment began with interferon beta-1b, followed by a switch to natalizumab in 2014 due to high disease activity. However, natalizumab was later changed to fingolimod because of the increased risk of progressive multifocal leukoencephalopathy (PML). In 2021, fingolimod was stopped due to persistent lymphopenia, and the patient transitioned to cladribine in 2023. Clinical, radiological, and laboratory evaluations were key in guiding these treatment changes.

Results/conclusion. Despite experiencing multiple relapses, progressive disability, and severe lymphopenia under fingolimod, the patient's disease has been well-controlled after starting cladribine, with no new relapses or active lesions on imaging. This case underscores the importance of adapting MS treatment to the patient's evolving needs, carefully weighing the benefits of high-efficacy therapies against the risk of complications. It also raises the question: what should be the next step in managing a patient like this, and how do we balance efficacy, safety, and long-term disease control?

Keywords: multiple sclerosis, adaptive treatment, high-efficacy therapies, interferon beta-1b, natalizumab, fingolimod, cladribine, lymphopenia, disease progression

P069. Rare complications in regional anesthesia: hematomyelia post rahianesthesia

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Introduction. Subarachnoid hemorrhage and spinal hematomas represent rare but severe complications of spinal anesthesia, particularly in patients receiving anticoagulant therapy.

Materials and methods. We present the case of a 77-year-old male with chronic atrial fibrillation on apixaban anticoagulation therapy, scheduled for elective inguinal herniorrhaphy under spinal anesthesia with 0.5% marcaine. The apixaban treatment was switched to enoxaparin preoperatively. However, the patient subsequently developed progressive neurological deficits, followed by cardiorespiratory arrest unresponsive to resuscitation.

Results. At 12 hours postoperatively, the patient developed paraplegic motor deficits (0/5 MRC score) with complete loss of tactile, thermal, and pain sensation below the T10 dermatome. Spinal MRI revealed

intramedullary hemorrhage (C6–T3) and signs of hemoglobin degradation at the T10 conus medullaris level. Lumbar puncture revealed hemorrhagic cerebrospinal fluid. On postoperative day 7, the patient developed bilateral T4 cutaneous anesthesia and brachial diparesis (2/5 MRC), and by postoperative day 9 required ICU transfer due to respiratory failure. On postoperative day 10, brain CT imaging revealed bilateral subarachnoid hemorrhage. Death occurred due to cardiorespiratory arrest unresponsive to resuscitation maneuvers. Necropsy revealed an extensive subarachnoid hemorrhage, both in the brain and in the medullary canal.

Conclusion. This case highlights the serious risks of anticoagulant therapy in combination with spinal anesthesia. For prompt diagnosis of hematomyelia, early detection of a specific spinal cord injury pattern is essential, which may involve various forms of myelopathy.

P070. Neurocysticercosis in disguise: amnesia, motor deficit and the silent parasite

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Introduction. The incidence of new-onset seizures in the adult population is estimated to be around 20–40 per 100,000 people per year. Adult-onset seizures require a bespoke approach due to the likelihood of being secondary to an identifiable cause, among which the most frequent are stroke, trauma, central nervous system infections, space-occupying lesions, metabolic disorders, and drugs.

Material and methods. We hereby present the case of a 71-year-old female patient who was referred to our department for a transitory episode of retrograde amnesia and right brachial deficit. The history is remarkable for hypertension, hyperlipidemia, and impaired glucose tolerance. The cerebral CT scan revealed multiple hypodensities in different vascular territories, thus raising suspicion of a cardioembolic transient ischemic attack. The subsequent diagnostic workup included evaluating potential emboligenic sources, supplemented by brain MRI. The latter revealed expansive lesions in both the supratentorial and infratentorial regions, compatible with either an infectious substrate (tuberculosis or parasitosis) or a neoplastic etiology. For further clarification, a lumbar puncture was performed, as well as a CT scan of the chest, abdomen, and pelvis.

Conclusions. After completing the diagnostic workup, we narrowed the probable hypotheses to cerebral parasitosis, with the initial symptoms occurring in the context of an epileptic seizure. Serology was positive for toxoplasmosis, but without corresponding findings in the CSF. The definitive diagnosis was made based on imaging findings that were highly suggestive of neurocysticercosis, with the patient meeting Del Brutto's criteria for a definitive diagnosis. Antiparasitic and corticosteroid therapy were initiated according to international guidelines, with a favorable clinical and imaging evolution.

Keywords: TIA, adult-onset seizures, neurocysticercosis

P071. Hypokinetic delirium associated with severe paraneoplastic hypercalcemia

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Introduction. Paraneoplastic neuroendocrine syndromes are metabolic disorders that can be associated with malignant or benign tumors. They present with a wide range of clinical signs and symptoms due to the production of abnormal hormones, proteins, and other substances.

Delirium can be associated with paraneoplastic neuroendocrine syndromes.

Methods and outcomes. An 81-year-old patient, with a medical history of hypertension and diabetes, presented with progressive deterioration, excessive sleepiness, and behavioral and cognitive disturbances, all occurring within the past week.

The clinical neurological examination revealed a drowsy and uncooperative patient, along with a bipyramidal syndrome accompanied by tetraplegia.

Magnetic resonance imaging (MRI) of the brain showed no abnormalities in the cerebral parenchyma, and electroencephalography did not reveal any epileptiform changes.

Non-contrast computed tomography (CT) of the chest, abdomen, and pelvis revealed multiple abdominal adenopathies.

Blood tests showed severe dyselectrolytemia, including hypercalcemia (Ca = 16.3 mg/dL), hypernatremia (Na = 159 mEq/L), and hypokalemia (K = 2.7 mEq/L). Hormonal screening revealed low parathyroid hormone (PTH = 7.22 pg/mL) and low circadian cortisol (Crt = 26.32 mcg/L). Infectious and autoimmune screening, as well as oncological markers, were negative.

Positron emission tomography–computed tomography (PET-CT) with 201 MBq F18-FDG confirmed the presence of metabolically active supra- and sub-diaphragmatic adenopathies. Histopathological examination of one adenopathy revealed tumoral tissue consistent with poorly differentiated carcinoma.

Therapy with denosumab monoclonal antibody was initiated, leading to an improvement in electrolyte imbalance and a favorable clinical evolution.

Conclusion. Although rare, paraneoplastic endocrine syndromes can cause metabolic disturbances that are treatable and should be considered in cases of delirium. Severe hypercalcemia is an indicator of a possible neoplasia, as seen in the case presented.

P072. Lupus, sepsis, and acute neurological decline: a case of PRES and RCVS

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Introduction. Neurological complications in systemic lupus erythematosus (SLE) represent a diagnostic and therapeutic challenge, especially in critically ill patients. This case highlights the diagnostic complexity of two potentially overlapping syndromes – posterior reversible encephalopathy syndrome (PRES) and reversible cerebral vasoconstriction syndrome (RCVS) – and underscores the importance of early recognition for optimal management.

Materials/methods. We present the case of a 38-year-old woman, recently diagnosed with severe active SLE, who developed acute neurological deterioration with psychomotor agitation, confusion, fever, and hypertensive crisis. An initial brain CT was deemed unremarkable; however, reevaluation of the same scan revealed previously missed cerebral vasoconstriction. The clinical picture evolved with signs of meningeal irritation. CSF analysis showed pleocytosis and low glucose levels, raising suspicion of bacterial meningitis, which was ruled out by BioFire panel and negative cultures. A subsequent analysis of the same CSF sample revealed markedly elevated red blood cell count, suggestive of subarachnoid hemorrhage. Simultaneously, MRSA endocarditis, bronchopneumonia, and sepsis were diagnosed, requiring ICU admission for respiratory and hemodynamic instability.

Results. The onset of new neurological deficits (aphasia, hemianopsia) prompted repeat brain CT, which showed bilateral parieto-occipital hypodensities, initially interpreted as watershed ischemia due to RCVS. Hemodynamic instability contraindicated nimodipine. The subsequent resolution of hypodensities indicated vasogenic edema, consistent with PRES. MRI findings ultimately supported the combined diagnoses of PRES and RCVS. Despite critical illness-associated myopathy and neuropathy, neurological recovery was slowly favorable.

Conclusion. This case highlights the difficulty of distinguishing PRES from RCVS, the importance of early recognition, and suggests a possible shared pathophysiological mechanism.

Keywords: systemic lupus erythematosus (SLE), posterior reversible encephalopathy syndrome (PRES), reversible cerebral vasoconstriction syndrome (RCVS), subarachnoid hemorrhage (SAH)

P073. What could be the cause behind an isolated bulbar syndrome with fulminant evolution in an elderly patient with vascular risk factors? A case report

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Introduction. Isolated bulbar syndrome is a neurological presentation that poses diagnostic challenges for many clinicians. To determine its etiology, multiple

paraclinical investigations are often required, which can delay definitive diagnosis and, implicitly, the initiation of treatment.

Methods. We illustrate the case of a 70-year-old male patient with a recent history of gastric sleeve surgery, ischemic stroke, and chemically converted atrial flutter a few years ago, without anticoagulant treatment at the time of admission. He presented to the emergency room with swallowing difficulties and dysarthria, symptoms that had progressively worsened over three days.

The fulminant evolution, with respiratory failure and acute vocal cord paralysis, required emergency tracheostomy. Facial diplegia subsequently emerged. At this point, the differential diagnosis included myasthenic crisis versus Guillain–Barré syndrome, and the patient was started on intravenous immunoglobulins.

The final diagnosis was established a few days after the completion of immunoglobulin therapy, following the result of anti-acetylcholine receptor antibody testing: a high-positive titer.

Conclusion. Onset with isolated bulbar syndrome is uncommon in myasthenic crisis, and even more so following bariatric surgery, for which no cases have previously been reported. A differential diagnosis including myasthenia gravis should be considered regardless of the patient's age, even in the absence of other typical signs such as limb muscle weakness or ocular involvement.

Keywords: myasthenia gravis, bulbar syndrome, bariatric surgery, fulminant evolution

P074. Paraneoplastic cerebellar degeneration in early detection of ovarian cancer: a case report

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Background. Paraneoplastic cerebellar degeneration (PCD) is a rare neurological syndrome associated with various cancers, with a predominance in lung and female genital tract cancers. Cerebellar syndrome typically presents with subacute onset and frequently precedes the diagnosis of the underlying tumor. Consequently, malignancies may be identified at an early stage, significantly improving the effectiveness of therapeutic interventions.

The presence of onconeural antibodies in a patient's serum represents an essential first diagnostic step, with anti-Yo antibodies being among the most frequently detected markers in cases linked to gynecological and breast cancers. However, identifying the underlying malignancy can be challenging and may require multiple interdisciplinary consultations, serological testing, and imaging studies.

Case presentation. This case involves a 49-year-old female patient with no significant prior medical history, presenting with a subacute onset of neurological symptoms that progressively worsened over five months. Her clinical picture included dysarthria, gait and coordination disturbances, as well as behavioral changes marked by significant mood instability and pronounced emotional fluctuations.

The patient demonstrated notable symptom improvement following a therapeutic regimen of intravenous methylprednisolone at 500 mg per day for three consecutive days. After comprehensive diagnostic evaluations, detection of anti-Yo antibodies confirmed the diagnosis of paraneoplastic cerebellar degeneration. Imaging studies subsequently revealed a mass in the left fallopian tube, and histopathological examination of the resected specimen confirmed high-grade serous carcinoma.

Conclusions. This case highlights the importance of persistent and thorough diagnostic workup for the early recognition of paraneoplastic cerebellar degeneration.

P075. Becker muscular dystrophy: clinical characteristics and current treatment approaches

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Becker muscular dystrophy (BMD) is a milder form of Duchenne muscular dystrophy. The incidence is approximately 1 in 30,000 live births. BMD is caused by abnormalities in the dystrophin gene, which encodes the muscle protein dystrophin. Dystrophin is a large filamentous protein of 3,685 residues and a molecular weight of 427 kDa. In BMD, abnormal but partially functional dystrophin is produced. It is an X-linked recessive disorder, mainly caused by exon deletions in the dystrophin gene (Xp21). This mutation results in a semifunctional dystrophin protein.

Case presentation. We present the case of two brothers, aged 27 and 35 years, who presented with complaints of myalgia, progressive muscle atrophy, and gait disturbances evolving over 3 and 6 years, respectively. The clinical assessment was challenging due to cognitive impairment, with recorded IQ levels of 44 and 58.

A key particularity of this case is the differential pattern of muscle involvement between the two siblings. One brother exhibited predominant scapular girdle involvement, whereas the other showed preferential pelvic girdle involvement. Both presented with a positive Gowers' sign and bilateral pseudohypertrophy of the posterior calf muscles. Notably, the patients reported no similar symptoms within their family; they have seven sisters and asymptomatic parents.

Laboratory findings showed significantly elevated muscle enzyme levels, while electrodiagnostic studies identified myopathic changes. Genetic testing confirmed an MLPA deletion in the DMD gene, consistent with a diagnosis of Becker muscular dystrophy.

Conclusion. New therapies have moved from preclinical studies to clinical trials, and BMD is now seen as a condition in which to evaluate non–mutation-specific therapeutic approaches for dystrophinopathies, including the synthetic steroid vamorolone, the histone deacetylase (HDAC) inhibitor givinostat, and the small molecule EDG-5506, a selective fast myosin inhibitor.

Moreover, the most promising therapeutic strategies currently in development – including gene transfer therapy using micro-/mini-dystrophin and exon-skipping approaches – aim to restore the expression of a partially functional dystrophin, similar to that observed in BMD.

Beyond pharmacological innovation, this progress must be paired with a supportive, multidisciplinary approach designed to enhance health-related quality of life. The care team should include physiotherapists, occupational therapists, and speech therapists.

Keywords: Becker muscular dystrophy, genetic testing, cognitive impairment, muscle atrophy, pelvic girdle, scapular girdle

P076. From carotid artery dissection to Moya-Moya disease: a misleading stroke diagnosis

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Moya-Moya is a rare, progressive arteriopathy affecting the large intracranial arteries, particularly the branches of the internal carotid artery, which leads to ischemic events or intracranial hemorrhage.

We present the case of a 45-year-old female patient, transferred from another clinic for sudden onset of distal right upper limb muscle weakness, 3 hours before admission. On neurological examination in the emergency room, the patient exhibited mild, non-disabling sensory-motor impairment of the distal right upper limb. Brain imaging revealed a tapered occlusion of the post-bulbar segment of the right internal carotid artery, highly suggestive of arterial dissection, as well as a severe stenosis of the intracranial segment of the left internal carotid artery. Subsequently, dual antiplatelet therapy was initiated for secondary prevention of stroke. Magnetic resonance imaging (MRI) showed multiple acute watershed ischemic lesions in the left hemisphere.

During hospitalization, extensive etiological investigations were conducted to rule out systemic vasculitis. A digital subtraction angiography (DSA) confirmed the findings of the CT angiography and additionally identified significant proximal stenosis of the M1 segment of the left middle cerebral artery and the A1 segment of the left anterior cerebral artery. Given the comprehensive etiological workup, a primary vasculopathy within the Moya-Moya disease spectrum was suspected.

Due to the occurrence of a minor ischemic event one month after discharge, the patient was considered to have significant cerebral hemodynamic insufficiency. Consequently, she was referred for extracranial-to-intracranial bypass surgery to improve cerebral hemodynamics.

This case highlights the diagnostic complexity of Moya-Moya vasculopathy, which can mimic other pathologies – in this instance, resembling carotid artery dissection.

P077. Huntington's disease in a patient with a pathogenic mutation in the HTT gene and an incompletely penetrant mutation in the SCA2 gene

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Introduction. Huntington's disease is a progressive, hereditary neurodegenerative disorder with autosomal dominant inheritance, leading to a gradual decline in motor, cognitive, and psychiatric functions. It is caused by a genetic mutation on chromosome 4, affecting the production of the huntingtin protein.

We present the case of a patient with a phenotype compatible with Huntington's disease, whose particularity lies in the identification of both a pathogenic trinucleotide CAG repeat expansion in the HTT gene and a pathogenic repeat expansion in the SCA2 gene with incomplete penetrance.

Materials and methods. We report the case of a 46-year-old female patient with a significant family history of a hereditary neurodegenerative disorder, who presented to the clinic with a gait disturbance characterized by imbalance, which had developed a year prior and had rapidly progressed. She also exhibited an executive dysfunction syndrome.

She underwent the following investigations: native brain MRI; genetic panel for spinocerebellar ataxias; and genetic testing for the HTT gene.

Conclusions. In the context of complex neuropsychiatric symptoms and clinical neurological findings at admission – including postural instability, ataxia, brainstem syndrome, pyramidal syndrome, and paresthetic syndrome – the primary challenge is the differential diagnosis between multiple forms of spinocerebellar ataxia and Huntington's disease.

This case particularly highlights the importance of an accurate diagnosis between Huntington's disease and SCA2 for appropriate treatment, prognosis, and genetic counseling.

Keywords: Huntington's disease, spinocerebellar ataxia (SCA2), neurodegenerative disorder, HTT gene, genetic testing, gait disturbance, imbalance, family history

P078. Atypical presentation of ICAD as hemilingual edema

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Introduction. Internal carotid artery dissection (ICAD) presents with diverse clinical features, ranging from asymptomatic cases or nonspecific head and neck pain to severe ischemic stroke. Common associated symptoms include local pain, Horner syndrome, pulsatile tinnitus, and cranial neuropathies.

Case presentation. We report a case of a 23-year-old man, previously healthy and a non-smoker, who presented with a four-day history of left-sided tongue swelling. He initially experienced occipital headache, left pulsatile tinnitus, and vague visual symptoms, which resolved prior to admission. He denied recent cervical trauma or substance use.

Neurological examination showed left hemilingual edema and mild dysarthria, with no other focal deficits. Laboratory workup, including thyroid function and vasculitis screening, was unremarkable. Contrast-enhanced MRI revealed a subintimal hematoma of the left internal carotid artery, extending from the skull base to the carotid canal (~2 cm), causing progressive luminal narrowing with up to 80% stenosis. A subacute ischemic stroke was also identified in the left middle cerebral artery territory. CTA confirmed the dissection from the C1 to proximal C2 segment, with 80–85% stenosis.

Treatment with antiplatelets and analgesics was initiated. At discharge, all symptoms had resolved except the tongue edema, which significantly improved over the following three months.

Conclusions. Unilateral tongue swelling should raise suspicion for ICAD. The exact mechanism remains unclear, but may involve postganglionic sympathetic fiber or hypoglossal nerve involvement, or central sympathetic dysfunction due to ischemia.

Keywords: carotid dissection, hemilingual edema, hypoglossal nerve

P079. Clinical, immunohistochemical and genetic diagnosis in Miyoshi myopathy – case report

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Introduction. Miyoshi myopathy is an autosomal recessive dysferlinopathy, characterized by preferential distal involvement of the posterior calf muscles, eventually extending to the anterior compartment, proximal muscles, and upper extremities. Respiratory failure is uncommon.

Materials/methods. A 30-year-old male patient presented to the clinic with suspected myotonic dystrophy type 1 for clinical reevaluation and muscle biopsy. The patient reported the onset of walking difficulties two years earlier, caused by muscle weakness that initially affected the left lower limb, and later became bilateral. He had no suggestive family history and had previously been investigated for subfertility.

Clinical examination did not reveal myotonia after prolonged contraction, repeated contraction-relaxation, or percussion. The patient could walk independently, but was unable to walk on tiptoes. CK levels were 3912 IU/L. Electromyography (EMG) revealed varying myopathic involvement in all muscle groups examined, and the absence of myotonic discharges.

Given the age of onset and predominant involvement of the posterior calf muscles, a muscle biopsy was performed, and immunohistochemical staining for dysferlin was requested, showing absence of dysferlin in the muscle sample. Genetic testing confirmed the diagnosis of Miyoshi myopathy.

Conclusions. Miyoshi myopathy presents with a distinctive clinical phenotype at onset, prior to the extensive involvement of other muscle groups. Although no curative treatment is currently available, genetic testing in myopathies is essential for better assessment of the functional and vital prognosis, as well as for reproductive counseling.

Keywords: Miyoshi myopathy, dysferlinopathy

P080. Sporadic inflammatory cerebral amyloid angiopathy in a patient with a history of obsessive-compulsive disorder: a case report

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Introduction. Cerebral amyloid angiopathy (CAA) is a group of disorders characterized by the deposition of amyloid β-peptide (Aβ) in the walls of cerebral and leptomeningeal vessels, in the absence of systemic amyloidosis. This can lead to vessel wall weakening, rupture, and cerebral hemorrhage. The sporadic form mainly affects the elderly. Cerebral amyloid angiopathy-related inflammation (CAA-RI) is a rare, aggressive variant with distinct radiological findings.

Case presentation. We present the case of a 44-year-old male, a smoker with a 14-year history of obsessive-compulsive disorder, currently on treatment with sertraline and levetiracetam, who presented the onset of his current illness in December 2024 with a generalized tonic-clonic seizure.

Cerebral MRI showed multiple nodular lesions, up to 16 × 7 mm, mainly in the superficial and deep white matter of the left hemisphere. The lesions exhibited hyperintensity on T2 FLAIR, hypointensity on T1, and no diffusion restriction. Some lesions showed peripheral or focal contrast enhancement, accompanied by digitiform edema, which significantly decreased following Solumedrol administration. Susceptibility-weighted imaging suggested the lesions represented microbleeds. Cerebral angiography revealed no pathological findings. Laboratory tests, including cerebrospinal fluid analysis, excluded autoimmune, infectious, or neoplastic processes. The patient showed no signs of systemic involvement.

Conclusion. This case suggests a rare form of sporadic inflammatory cerebral amyloid angiopathy (CAA-I). Considering the patient's young age and atypical presentation, a cerebral biopsy is recommended for definitive diagnosis, along with genetic testing to assess possible hereditary forms.

Keywords: cerebral amyloid angiopathy, amyloid β-peptide (Aβ), generalized tonic-clonic seizure

P081. A case of sequential anterior optic neuritis in neuroborreliosis associated with monoclonal gammopathy

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Introduction. Bilateral anterior optic neuritis is a rare manifestation of neuroborreliosis, and its association with monoclonal gammopathy is extremely uncommon in the medical literature.

Materials and methods. We present the case of a 46-year-old patient who initially experienced decreased visual acuity in the right eye in 2024. Ophthalmological and neuroimaging evaluations (brain MRI) did not reveal any relevant structural abnormalities, while serological tests confirmed the presence of anti-Borrelia burgdorferi antibodies (weakly positive IgM, positive IgG by confirmatory testing). Anti-aquaporin 4 and anti-MOG antibodies were negative.

The patient was initially treated with corticosteroids and antibiotic therapy (ceftriaxone 2 × 1 g/day) for 14 days, with no significant clinical improvement. In February 2025, he presented with decreased visual acuity in the left eye. A repeated brain MRI showed findings suggestive of right optic neuritis, and lumbar puncture revealed mild proteinorachia, with confirmation of neuroborreliosis through intrathecal synthesis of Borrelia-specific IgG. Serum protein electrophoresis indicated the presence of a possible monoclonal gammopathy. Ophthalmological examination revealed right optic atrophy.

Combined treatment with ceftriaxone (2 × 2 g/day for 28 days) and corticosteroids was initiated, with no significant improvement in the neurological status.

Conclusions. This case highlights an atypical presentation of neuroborreliosis with progressive bilateral optic involvement and an associated monoclonal gammopathy. The lack of favorable response to antibiotic therapy suggests a complex pathogenic mechanism. Further studies are needed to better understand the relationship between neuroborreliosis, optic neuritis, and monoclonal gammopathies, as well as to optimize therapeutic strategies in such cases.

Keywords: monoclonal gammopathy, optic neuritis, neuroborreliosis

P082. Status epilepticus in autoimmune encephalitis with negative MRI findings throughout disease course

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Introduction/objective. Autoimmune encephalitis is an immune-mediated inflammatory disorder of the central nervous system, with acute or subacute onset, often presenting with seizures, behavioral changes, and focal neurological deficits. Although anti–glutamic acid decarboxylase (anti-GAD) antibodies are rarely detected, they can be associated with severe forms of the disease, including pharmacoresistant epilepsy and incomplete response to treatment.

We report a case of anti-GAD–associated autoimmune encephalitis presenting with status epilepticus and persistently normal brain MRI.

Materials and methods. A 27-year-old woman was admitted with meningitic symptoms and developed recurrent focal motor seizures with secondary generalization. Her condition progressed to spastic tetraparesis, expressive aphasia, and ataxia. CSF analysis revealed isolated protein elevation. Brain MRI remained normal throughout. EEG showed left temporal epileptiform discharges. Autoimmune and infectious panels were negative, except for positive anti-GAD antibodies (25.52 UI/mL).

Results/conclusions. Seizures were refractory to levetiracetam, carbamazepine, and phenytoin. The patient required intensive care with mechanical ventilation and sedation. She received five courses of intravenous immunoglobulins, five plasmapheresis sessions, and lacosamide was added. Clinical improvement was gradual. She was discharged in a stable neurological condition under combined antiepileptic and immunomodulatory therapy.

This case illustrates a severe presentation of anti-GAD autoimmune encephalitis with status epilepticus and no MRI abnormalities. It highlights the importance of considering autoimmune etiologies even in the absence of radiological findings, and the need for early immunotherapy to improve outcomes.

Keywords: autoimmune encephalitis, status epilepticus, anti-GAD, immunotherapy, pharmacoresistant epilepsy, negative MRI

P083. Ischemic stroke in a patient with systemic lupus erythematosus – possible mechanism when the patient is already on stroke prevention therapy

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Introduction. Stroke is relatively frequent in patients with systemic lupus erythematosus (SLE), affecting 3–20%, especially in younger individuals. Pathogenesis involves vasculitis, a prothrombotic state due to antiphospholipid antibodies (aPL), endothelial dysfunction, steroid use, and early atherosclerosis.

Methods. We present the case of a 35-year-old, non-smoker male with SLE who developed left-sided hemiplegia, central facial palsy, and hypoesthesia. Prior to this, he had experienced transient left-sided weakness over the previous two weeks. Initially, the symptoms lasted five minutes with full recovery. During the first 12 hours of hospitalization, he experienced three more transient episodes, which culminated in permanent deficits.

Repeated brain imaging showed an acute stroke in the deep right MCA territory with M1 segment stenosis, as well as occlusion of the left internal carotid artery. The patient was undergoing treatment with acenocoumarol for antiphospholipid syndrome, with an INR that was above the therapeutic range, and was also taking a high-dose statin.

Conclusion. Accelerated atherosclerosis due to chronic autoimmune inflammation was considered the main cause of his large-vessel steno-occlusive lesions, given that he developed the stroke under anticoagulant therapy with a supratherapeutic INR and full-dose statin. Antiplatelet therapy was added, along with continued anticoagulation, statins, antihypertensives, and immunosuppressants.

Discussion. SLE-accelerated atherosclerosis results from chronic inflammation and is a leading cause of mortality in SLE patients. It goes beyond classical cardiovascular risk factors, complicating both diagnosis and management.

Keywords: atherosclerosis, systemic lupus erythematosus, stroke

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P084. Intractable chronic cough and internal jugular vein ectasia – an unexpected connection

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Introduction. Chronic cough is a common symptom with multiple causes, predominantly respiratory or gastroesophageal. In rare cases, a vascular anomaly such as internal jugular vein ectasia may act as a triggering factor. We report the case of a patient with paroxysmal chronic cough associated with right jugular ectasia, which responded favorably to anticonvulsant therapy.

Materials and methods. We present the case of a 62-year-old female patient with a pulmonary, cardiovascular, and neurological history, and a two-year history of daily episodes of severe, non-productive cough unresponsive to standard symptomatic treatments. ENT, pulmonary, and gastroenterological evaluations did not identify a specific organic cause. Cervical CT with contrast revealed a fusiform ectasia of the right internal jugular vein (area of 2.1 cm^2 compared to the left side, 0.4 cm^2).

Neurological examination was unremarkable, with no cranial nerve deficits. Based on the hypothesis of vagal irritation, treatment with carbamazepine was initiated, leading to significant improvement.

Conclusions. This case suggests a possible link between jugular ectasia and chronic neuropathic cough. Although rare, this etiology should be considered in chronic cough syndromes without an apparent cause. Neuropathic cough is underdiagnosed and may benefit from anticonvulsant treatment, while the identification of atypical mechanisms requires a multidisciplinary approach.

Keywords: chronic cough, jugular ectasia, sensory neuropathy, vascular compression, carbamazepine

P085. Persistent trigeminal artery – a chance for a patient with Takayasu arteritis

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Introduction. Takayasu arteritis (TA) is a large-vessel vasculitis predominantly affecting the aorta and its major branches, leading to vessel narrowing and subsequent complications arising from both the disease's pathogenesis and the effects of immunosuppressive therapy. This report presents a case of a patient with TA and highlights our clinical experience in its management.

Methods. A 36-year-old female patient with a history of TA, Cushing syndrome, hypertension, dyslipidemia, diabetes mellitus, and a recent ischemic stroke presented to our clinic for follow-up evaluation. Her medical history included a nine-year delay in diagnosis, with symptoms such as arm and leg muscle pain, leg claudication, cold extremities, dizziness, headaches, and several episodes of falls with loss of consciousness.

Clinical examination revealed absent bilateral radial artery pulses, right-sided facial numbness, and slight pronation of the right arm.

Results. Computed tomography angiography (CTA) of the upper aortic branches showed 55% stenosis in the middle third of the right common carotid artery (CCA) and the right vertebral artery, bilateral subclavian artery occlusion, left CCA occlusion, and a persistent trigeminal artery. CTA of the thoraco-abdominal aorta demonstrated 65% stenosis of the distal descending aorta and 40% stenosis of the visceral abdominal arteries.

Conclusion. In Takayasu arteritis, arterial stenosis and occlusion require careful clinical and imaging monitoring. In this case, the patient's imaging results were stable compared to the previous examination, and no treatment changes were recommended.

The particularity of this case is the persistent left trigeminal artery, which, in the context of left CCA occlusion, permitted perfusion of the left carotid brain territory.

Keywords: Takayasu arteritis, large-vessel vasculitis, arterial stenosis, arterial occlusion, persistent trigeminal artery, computed tomography angiography (CTA), clinical and imaging monitoring, vascular complications

P086. Searching for hidden malignancies: a case of anti-Hu antibodies encephalitis and neuropathy

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Introduction. Anti-Hu encephalitis is a rare form of paraneoplastic syndrome, most frequently associated with small cell lung carcinoma, though it can accompany various other malignancies. In a subset of patients, however, standard neoplasia screening may return negative, complicating the diagnosis.

Materials and methods. We present the clinical case of a 73-year-old female patient, a former smoker with no prior history of cancer, who presented to our clinic with involuntary myoclonic movements, confusion with visual hallucinations, subacute cognitive impairment, progressive tetraparesis, short-term memory loss, and a witnessed seizure.

Results. Upon initial neurological evaluation, the patient exhibited severe confusion, speech disturbances, and tetraparesis with ataxia in all four limbs. Cerebral CT and MRI scans showed no abnormalities. Lumbar puncture revealed normal glucose and protein levels, but showed pleocytosis. Nerve conduction studies indicated severe axonal polyneuropathy.

Extensive immunological testing revealed strong positivity for anti-Hu antibodies. Gradual improvement in both peripheral function and mental status was noted following intravenous corticosteroid therapy.

Conclusions. The neurological outcome of paraneoplastic encephalitis depends on timely diagnosis and treatment of the underlying malignancy. Given the rarity and complexity of such presentations, a multidisciplinary approach is essential for optimal management.

Keywords: anti-Hu antibodies, encephalitis, paraneoplastic syndrome, neuropathy, polyneuropathy, corticosteroids, malignancy

P087. Endovascular management of bilateral internal carotid artery dissection: a rare case of self-induced cervical crepitus leading to ischemic events

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Introduction. Bilateral internal carotid artery (ICA) dissection is an uncommon yet potentially life-threatening condition in younger individuals, representing 2–10% of all carotid dissections. It can present with diverse symptoms and may be triggered by various factors, including trauma.

Materials and methods. We report the case of a 40-year-old male with a medical history of arterial hypertension who presented with occipital headache following self-induced bilateral cervical crepitus. He experienced transient neurological events, including left-sided sensory impairment and left monocular vision loss, followed by multiple episodes of right-sided sensory disturbances and speech impairments.

An initial cerebral CT scan was unremarkable. Doppler ultrasonography of the cervical-cerebral arteries showed high-resistance flow signals in both ICAs. Cerebral and cervical MRI revealed millimetric, bilateral, non-enhancing hyperintensities on DWI and FLAIR sequences, suggestive of ischemia. Additionally, MRI identified intimal flaps and intramural hematomas in the cervical segments of both ICAs, confirming bilateral ICA dissection. CT angiography of the supraaortic trunks showed subocclusive stenosis of both ICAs, with left intracranial low flow.

Results. The patient underwent digital subtraction angiography (DSA) of the cervical-cerebral arteries and stenting of the left ICA. Postprocedural complications included arterial hypotension, right-sided hemiparesis, and aphasia, with complete recovery over the following day.

Conclusion. We present this case to emphasize the importance of a thorough imaging workup in young patients presenting with stroke symptoms. Furthermore, we highlight the value of multidisciplinary team management. Although conservative therapy remains the primary approach, endovascular treatment may be required in selected cases with hemodynamic impairment to prevent further cerebrovascular events.

Keywords: carotid artery dissection, cervical crepitus, ischemic stroke, endovascular therapy, internal carotid artery, stenting, imaging workup, multidisciplinary management

P088. Miller Fisher syndrome: a stroke mimic

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Background. Polyneuropathies are dysfunctions in multiple nerves and are broadly categorized as either axonal or demyelinating. Demyelinating neuropathies include toxic, hereditary, and immune-mediated etiologies. The immune-mediated forms can further be classified as acute or chronic, depending on onset.

Acute immune-mediated demyelinating polyneuropathies (AIDP) fall within the spectrum of Guillain–Barré syndrome (GBS), named after the French physicians who described it. This report focuses on Miller Fisher syndrome (MFS), a rare variant of GBS. First described by James Collier in 1932, MFS is defined by a triad of ophthalmoplegia, ataxia, and areflexia.

Case presentation. A 55-year-old patient with a known history of Hodgkin's lymphoma since 2005, non-compliant with recommended chemotherapy and with no history of toxic substance use, presented to the emergency department with dizziness, gait disturbances, diplopia, dysphagia, and dysphonia. The symptoms had worsened over the 2–3 days prior to admission.

Retrospectively, onset was subacute, starting 3–4 weeks before hospitalization with cervical radicular pain and paresthesia in all four limbs. Emergency cranial CT, extracranial Doppler ultrasound, brain MRI, and TOF MR angiography revealed no findings suggestive of ischemic cerebrovascular pathology.

Within 24 hours, the patient's symptoms worsened with motor deficits in all limbs, areflexia, and hypoglossal nerve palsy. Cerebrospinal fluid analysis revealed albuminocytologic dissociation.

Conclusion. We present the case of a patient with a 20-year history of untreated Hodgkin's lymphoma, who developed symptoms suggestive of ischemic stroke in the vertebrobasilar territory. However, the subacute onset, rapid clinical deterioration despite corticosteroid and stroke treatment, albuminocytologic dissociation, and favorable response to immunotherapy point toward an underlying acute demyelinating neuropathy, consistent with Miller Fisher syndrome.

Keywords: polyneuropathy, ataxia, ophthalmoplegia, areflexia, Miller Fisher syndrome, stroke mimic

P089. The unrelenting twitch: a rare case of stroke-induced epilepsia partialis continua

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Background. Epilepsia partialis continua (EPC) is a rare form of focal motor status epilepticus, often treatment-resistant and associated with structural, metabolic, or immunologic cerebral pathology. We present a case of refractory focal motor seizures secondary to multiple acute and subacute ischemic strokes.

Case presentation. A 77-year-old patient, recently discharged from our clinic after a minor left middle cerebral artery (MCA) cardioembolic stroke, was readmitted with acutely worsened right hemiparesis. The initial workup excluded a recurrent stroke, but within 24 hours, the patient developed right focal clonic and myoclonic seizures without impairment of consciousness.

EEG showed isolated and grouped delta activity over the left frontocentral region, fulfilling criteria for epilepsia partialis continua. The involuntary movements failed to respond to intravenous diazepam, requiring escalation to multiple antiepileptic agents. Seizure frequency and duration slowly declined, with full remission after two weeks of treatment.

Brain MRI revealed severe small-vessel disease and multifocal ischemic strokes of varying ages in infra- and supratentorial regions. CSF analysis and extensive immunological screening were unremarkable. Laboratory studies revealed an inflammatory syndrome. Thoracic CT was compatible with pneumonia.

The subsequent course was unfavorable, with infectious complications unresponsive to antibiotic therapy and supportive measures, leading to septic shock, coma, and death.

Conclusion. Epilepsia partialis continua is a rare complication of acute stroke. Response to treatment is often poor, and seizure control is challenging. Frequently resulting in prolonged hospitalization and secondary complications, EPC is associated with increased disability and mortality, as demonstrated by this case.

Keywords: epilepsia partialis continua, focal motor status epilepticus, ischemic stroke, small-vessel disease, EEG, pneumonia, refractory seizures, disability, mortality

P090. The unrelenting twitch: a rare case of stroke-induced epilepsia partialis continua

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Introduction. Most ischemic strokes result from cardioembolism, large-vessel atherothromboembolism, or small-vessel disease; however, up to 35% remain cryptogenic when no clear etiology is identified.

Case presentation. We present the case of a 54-year-old patient with ischemic heart disease, myocardial infarction 14 years prior, chronic heart failure (NYHA II), hypertension, and dyslipidemia. His home treatment included antiplatelets, antihypertensives, beta-blockers, and statins.

He presented with global aphasia and right hemiplegia, with symptom onset 50 minutes before admission. A CT scan showed no established ischemic lesions, while CT angiography revealed an M1 occlusion of the left middle cerebral artery (MCA). The patient underwent intravenous thrombolysis and mechanical thrombectomy, resulting in partial recanalization, with the inferior branch remaining occluded.

A mandatory cardiovascular evaluation was performed. Transthoracic echocardiography revealed an apical thrombus within an aneurysmal region of the left ventricle. Cervicocerebral Doppler ultrasound showed no significant abnormalities. A Holter ECG confirmed sinus rhythm. Additionally, diabetes mellitus was diagnosed during hospitalization.

Conclusion. The patient showed significant improvement following revascularization, with only mild residual receptive aphasia. Dual antiplatelet therapy was initiated and later transitioned to oral anticoagulation upon confirmation of the left ventricular thrombus.

This case highlights the importance of comprehensive cardiac evaluation in ischemic stroke, especially when other investigations are inconclusive. Intracavitary ventricular thrombi represent the second most common cause of cardioembolic stroke, following atrial fibrillation.

It reinforces the necessity of morphological studies in ischemic stroke workup and illustrates the critical role of prompt reperfusion therapy in improving neurological outcomes.

Keywords: cardioembolic stroke, intracavitary thrombus, echocardiography

P091. Atypical complex hereditary spastic paraplegia in a patient with heterozygous SPG11 mutation and a YARS variant: coincidence or digenic mechanism?

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Introduction. Spastic paraplegia type 11 (SPG11) is among the most common autosomal recessive forms of hereditary spastic paraplegia (HSP). It is typically characterized by adolescent-onset gait abnormalities, progressive lower limb spasticity, and frequent cognitive impairment. While mild sensory neuropathy may occur, it is not usually a dominant feature.

We present a patient with a compatible clinical phenotype but preserved cognition, significant axonal polyneuropathy, and only a single heterozygous SPG11 pathogenic variant. Additionally, a second variant of uncertain significance (VUS) was identified in the YARS gene. The aim is to explore a possible additive or digenic genetic contribution.

Material and methods. We report the case of a 47-year-old Caucasian male with gait disturbance beginning at age 14. He was followed over a period of 20 years with regular clinical assessments, repeated electrophysiology (EMG), brain and spinal MRI, and an extended hereditary neuropathy gene panel.

Conclusion. His condition progressed with worsening gait disturbance and spasticity, and at age 29, sensory symptoms emerged. Electrophysiology revealed slowly progressive tetramelic axonal sensory polyneuropathy. Genetic testing identified a heterozygous pathogenic SPG11 variant (c.6091C>T, p.Arg2031*) and a YARS VUS (c.104A>G, p.Lys348Glu).

A recent examination showed spastic-ataxic gait, tetramelic ataxia (lower > upper limbs), abolished vibration sense, Charcot arthropathy, horizontal diplopic nystagmus, bilateral hypoacusis, but no cognitive decline. The clinical picture raises suspicion of a possible modifying role of the YARS variant in the expression of the complex phenotype.

This case supports the importance of phenotype-genotype correlation and considering multiple genetic variants in hereditary neurological conditions.

Keywords: hereditary spastic paraplegia, SPG11, YARS, axonal polyneuropathy, digenic mechanism, phenotype-genotype correlation

P092. Cancer-associated ischemic stroke: a case report

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Introduction. Cancer-associated ischemic stroke is a distinct clinical entity characterized by stroke occurrence in the setting of active malignancy, often in the absence of conventional cardiovascular risk factors.

Case presentation. We present the case of a 46-year-old female, a non-smoker with no significant medical history, who was admitted to the Emergency Department of the Emergency County Hospital of Targu Mures due to pronounced edema in the left upper and lower limbs, accompanied by pain and limited mobility. Blood tests revealed significantly elevated D-dimer levels (>5000 ng/mL), raising suspicion of venous thromboembolism. A venous Doppler ultrasound was performed, which identified multiple deep vein thromboses, in addition to pulmonary embolism and a macronodular mass in the right lower lobe, as confirmed by native thoracic CT scan and CT pulmonary angiography.

During her hospitalization in the Clinic of Internal Medicine, the patient was treated with low-molecular-weight heparin (LMWH). However, after several days, she developed sudden left hemiparesis. A brain CT scan revealed multiple space-occupying lesions in the left cerebral and cerebellar hemispheres. Further evaluation with brain MRI showed two T2-FLAIR hyperintense lesions in the right cortico-subcortical region, suggestive of acute ischemic stroke. She was transferred to our clinic for further investigations. Histopathological examination of a lymph node biopsy indicated the presence of lung adenocarcinoma.

Conclusion. Although rare, acute ischemic stroke associated with a malignancy-induced hypercoagulable state should not be underestimated, particularly in young patients without obvious risk factors. Prompt diagnosis and management are crucial in such cases.

Keywords: ischemic stroke, lung adenocarcinoma

P093. Atypical onset of progressive multifocal leukoencephalopathy: beyond the classic pathophysiology of HIV

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Introduction/objective. Progressive multifocal leukoencephalopathy (PML) is a severe demyelinating condition caused by reactivation of the JC virus, typically seen in immunosuppressed patients. Although PML associated with HIV is less common in the antiretroviral era, it can pose significant diagnostic challenges, particularly in atypical or acute-onset presentations.

Material and methods. We present the case of a 60-year-old woman with no prior HIV diagnosis, admitted with flaccid left hemiplegia, global aphasia, and cortical blindness. The acute clinical presentation led to an initial suspicion of stroke, but the subsequent investigations raised the suspicion of a possible infiltrative brain lesion. Brain MRI with intravenous contrast revealed extensive subcortical fronto-parieto-temporal lesions in the right hemisphere, extending through the splenium of the corpus callosum to the contralateral side. CSF PCR was positive for JC virus DNA. HIV testing returned reactive. A stereotactic brain biopsy provided an extemporaneous result suggesting HIV encephalitis.

Results/conclusion. Although current histology does not demonstrate classic demyelination, the radiological appearance, CSF confirmation of JC virus, and the fulminant clinical evolution support the diagnosis of PML. This case underscores the complexity of differentiating PML from HIV encephalitis in advanced immunosuppression and highlights the importance of HIV testing in patients with atypical white matter lesions.

Keywords: PML, HIV, JC virus, encephalopathy, stroke mimic, brain biopsy

P094. A case of hypokalemic periodic paralysis

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Hypokalemic periodic paralysis is an autosomal dominant disease mainly affecting male patients. It is a rare channelopathy leading to recurrent episodes of muscle weakness, which can last from several hours to days, and is associated with low serum potassium levels.

Case presentation. We report a case of hypokalemic periodic paralysis in a 25-year-old male patient, a smoker, diagnosed with stage II hypertension, pseudomicronodular goiter, and hypercholesterolemia, with a history of recurrent episodes of muscle weakness following dehydration or a high-carbohydrate diet beginning at the age of 14. The episodes usually lasted 1–2 hours and resolved spontaneously. He was admitted to the emergency department in November 2024 for a new episode of generalized hypotonia. Neurological examination revealed flaccid quadriparesis, with no coordination or sensory deficits. Blood tests were within normal range, except for a low serum potassium level (2.8 mmol/L), and EMG revealed pathological myopathic motor unit action potentials. The patient fully recovered after administration of antialdosteronic therapy and potassium supplements.

Conclusion. By excluding renal and adrenal insufficiency, thyroid dysfunction, and aldosteronism, the clinical pattern and EMG findings – together with underlying hypokalemia – led to the probable diagnosis of primary hypokalemic periodic paralysis.

Keywords: hypokalemia, periodic paralysis, muscle weakness, EMG, potassium, channelopathy

P095. Diagnosis and management of Huntington's chorea: modern approaches and therapeutic challenges

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Introduction/objective. Huntington's chorea is a rare, genetically transmitted neurodegenerative disease characterized by chorea, psychiatric symptoms, and cognitive decline. Recent advances in understanding pathogenic mechanisms and diagnostic technologies have opened significant perspectives for a modern, personalized approach to managing this condition.

Materials and methods. We present the case of a 51-year-old female patient, genetically diagnosed with Huntington's chorea (41 CAG repeats) and with a relevant family history (father and brother affected). She sought neurological evaluation due to persistent choreic symptoms despite treatment with amantadine 100 mg twice daily.

Results. Initiation of therapy with tetrabenazine was considered; however, psychiatric and psychological evaluations revealed severe depressive disorder, representing a contraindication for this medication. Under these circumstances, treatment with tiapride was initiated and gradually increased, with an initial slight improvement. Nevertheless, worsening of choreic symptoms at the maximum tolerated dose required gradual discontinuation. Subsequently, pimozide therapy was started, leading to significant improvement in motor symptoms. In addition to pharmacological treatment, physiotherapy, occupational therapy, and psychological counseling were recommended to manage the emotional and cognitive impact.

Conclusions. This case highlights the complexity of therapeutic management in Huntington's chorea, requiring a multidisciplinary approach tailored to each patient's specific characteristics. Optimal treatment choice depends on close monitoring and collaboration b**etween the neurologist, psychiatrist, and family.**

Keywords: Huntington's chorea, therapeutic management, multidisciplinary approach, therapeutic challenges

P096. Systemic and cerebral embolisms of undetermined source secondary to a possible neoplasia – reflections on a case

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Introduction. Almost one-quarter of patients with ischemic stroke have an embolic mechanism with undetermined source (ESUS). Between 2% and 10% of patients are diagnosed with neoplasia within the first year after stroke.

Methods and results. We present the case of a 71-year-old female patient, with a history of hypertension and dyslipidemia, admitted for ictal-onset language disorder.

Initial neurological examination revealed motor aphasia. Laboratory investigations showed thrombocytopenia ($42 \times 10^3/\mu L$), low fibrinogen (107 mg/dL), and dynamic changes in cardiac troponin and NT-proBNP levels.

Brain MRI revealed multiple bihemispheric acute ischemic strokes.

Transthoracic echocardiography identified left atrial dilation, while transesophageal echocardiography revealed a round, oval formation attached to the aortic valve, suggestive of infective or non-infective endocarditis.

Tumor markers and infectious screening were negative.

Systemic CT imaging (chest, abdomen, pelvis) showed subdiaphragmatic adenopathies, and renal and splenic infarctions.

Coronary angiography revealed thrombotic occlusion of the anterior descending artery.

Given the presence of multiple systemic infarcts (cerebral, myocardial, renal, and splenic) with an embolic pattern, anticoagulant therapy was initiated.

The patient's condition deteriorated, with the appearance of a generalized pruritic erythematous rash (suggestive of cutaneous lymphoma), worsening neurological status, and repeated brain MRIs showing new acute ischemic lesions.

Conclusion. In patients with ischemic stroke, recurrent events, and systemic infarcts, ESUS should be considered. This case may be classified as marantic endocarditis associated with an underlying hematological malignancy. A multidisciplinary approach with dynamic follow-up is essential for optimal management.

Keywords: stroke, neoplasia, ESUS, recurrence, systemic infarctions

P097. Pancerebellar syndrome associated with acute varicella-zoster virus infection

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Introduction. Acute cerebellar ataxia (ACA) is a known complication of varicella-zoster virus (VZV) infection, commonly reported in children but rare in adults.

Methods and results. A 21-year-old female patient presented with dysarthria, postural instability, headache, nausea, and vomiting, which had developed subacutely over 96 hours. These symptoms followed a systemic VZV infection that began 10 days prior.

Clinical examination revealed a generalized rash in the hematic crust phase. Neurological examination identified an asymmetric pancerebellar syndrome.

Etiological confirmation was achieved through serology, showing positive IgG and IgM anti-VZV antibodies.

Cerebrospinal fluid (CSF) analysis revealed mild pleocytosis (16 cells/µL, 100% mononuclear), with normal glucose and protein levels. A multiplex PCR panel for viruses, bacteria, and fungi was negative, indicating an aseptic meningitis pattern.

Brain MRI with contrast showed no parenchymal abnormalities.

Treatment with intravenous acyclovir and methylprednisolone led to progressive clinical improvement and remission of neurological deficits.

Conclusions and discussion. Although ACA is a frequent neurological complication of VZV in children, it remains rare in adults. The condition can present with variable clinical features and prognosis, making recognition and early treatment essential.

Keywords: pancerebellar syndrome, ataxia, VZV infection, aseptic meningitis

P098. Ectromelic right brachial monoparesis of central origin – "peripheral neuropathy mimic"

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Acute ischemic stroke can occur through three main mechanisms: artery-to-artery embolism, heart-to-artery embolism, and hemodynamic (local or systemic). The clinical presentation of stroke, characterized by sudden focal neurological deficits, varies depending on the affected vascular territory. While motor deficits typically involve an entire hemibody, some cases present with highly limited deficits, mimicking peripheral nerve involvement.

We highlight the case of a 62-year-old man with cardiovascular risk factors and known permanent atrial fibrillation, who presented with clinical features suggestive of a right-sided radial nerve paresis, noticed on waking in the morning. A brain computed tomography scan within 12 hours of onset showed no acute lesions, raising suspicion of a peripheral cause, possibly linked to pre-existing peripheral neuropathy. However, multiple cardiovascular risk factors and a history of stroke prompted further investigation. Therefore, brain MRI revealed a small subacute ischemic stroke in the left frontal corona radiata. Supra-aortic trunk angiography identified an asymptomatic 60% stenosis of the right internal carotid artery.

The particularity of the case lies not only in the limited neurological presentation, but also in the therapeutic approach, which involved switching one direct oral anticoagulant class for another and adding antiplatelet therapy.

This case underscores the critical role of medical history and neurological examination in guiding accurate diagnosis, preventing confusion between central and peripheral pathologies, particularly in patients with cardiovascular risk factors.

Keywords: stroke, peripheral neuropathy mimic

P099. When smoke leaves scars in the brain: a case of toxic leukoencephalopathy following carbon monoxide poisoning

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Introduction. Carbon monoxide (CO) poisoning remains one of the most frequent causes of accidental toxic exposure with lethal potential worldwide. Although its incidence has declined in developed countries in recent decades, cases still occur, particularly in the context of improper stove use. Neurological effects may range from mild symptoms to severe toxic leukoencephalopathy, with either immediate or delayed onset.

Method. We present the case of a 73-year-old female with chronic alcohol use, found by emergency services in a heavily smoke-filled room due to a malfunctioning wood-burning stove. The patient developed, in approximately three weeks, a confusional syndrome, tetraparesis, and epileptic seizures with unclear onset and secondary generalization. The diagnostic workup included a neurological examination, laboratory testing (including toxicology), electroencephalography, cerebral computed tomography (CT), and magnetic resonance imaging (MRI).

Results. Laboratory investigations revealed severe anemia (Hb = 7.4 g/dL), while arterial blood gas analysis showed no significant abnormalities, likely due to delayed sampling. Cerebral CT revealed bilateral, symmetrical digitiform hypodensities in the supratentorial white matter, sparing the basal ganglia and thalami. These findings were confirmed by MRI, which demonstrated T2 and FLAIR hyperintensities in the fronto-temporo-parieto-occipital white matter and external capsules, consistent with toxic leukoencephalopathy.

Conclusions. This case highlights a rare and severe form of injury secondary to prolonged CO exposure, with the notable feature of basal ganglia sparing, possibly reflecting subacute intoxication. Early diagnosis and a multidisciplinary approach (neurology, toxicology, radiology, intensive care), alongside timely management of cerebral edema, are essential. The prognosis remains poor, with a high risk of long-term disabling neurological sequelae.

P100. Pontine and extrapontine myelinolysis presenting as acute parkinsonism – a case report

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Introduction. Osmotic demyelination syndrome is an infrequent but serious complication that may arise from the rapid correction of severe and prolonged hyponatremia. The most commonly affected region is the cerebral pons, typically presenting with severe bipyramidal syndrome. Extrapontine lesions, such as those involving the thalamus or cerebellum, can rarely be found and can lead to atypical clinical manifestations. Seldom, when the basal ganglia are affected, patients may present with signs and symptoms resembling parkinsonism.

Materials and methods. A 56-year-old male patient, heavy alcohol drinker, was admitted to the neurology department for masked face, dysarthria and dysphagia, severe generalised rigidity, hypo- and bradykinesia, parkinsonian gait, and walking achieved with great difficulty. The patient did not present with any pyramidal tract signs. The symptoms occurred abruptly, following rapid correction of severe hyponatremia after excessive, prolonged vomiting. MRI revealed typical features of central pontine and extrapontine myelinolysis. The patient's clinical status improved gradually after supportive treatment, with complete remission of the symptoms at the 6-month follow-up.

Conclusions. Osmotic demyelination syndrome constitutes a rare underlying cause of clinical acute-onset parkinsonism, and although the prognosis is poor, patients can make a full recovery.

Keywords: pontine myelinolysis, extrapontine myelinolysis, hyponatremia, parkinsonism, osmotic demyelination syndrome

P101. Dabigatran reversal – the light at the end of the tunnel for patients with acute ischemic stroke

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Introduction. Many patients who have had a stroke caused by non-valvular atrial fibrillation are prescribed Dabigatran, a reversible direct thrombin inhibitor. Its reversal agent, Idarucizumab, may sometimes be the only factor enabling timely revascularization and preventing further neurological damage.

Method. A 45-year-old woman with a history of recurrent ischemic strokes presented with worsening motor deficits in the right limbs (NIHSS score of 8). At the time of admission, she was undergoing treatment with Dabigatran.

Results. In the emergency department, a CT scan showed no acute lesions. CT angiography confirmed middle cerebral artery (MCA) occlusion, first documented in 2023. Brain MRI revealed an acute internal capsule lesion. The patient received Idarucizumab, followed by intravenous thrombolysis. However, her NIHSS score remained unchanged. During hospitalization, she experienced another stroke recurrence. After two weeks, she was transferred to a motor rehabilitation center, where she began walking again with unilateral aid.

Conclusion. This case highlights the importance of ensuring access to Dabigatran reversal agents in all acute stroke units. As the use of novel oral anticoagulants becomes increasingly common, the risk of stroke recurrence – although low – warrants preparedness for rapid intervention.

Keywords: reversal agent, atrial fibrillation, revascularization, dabigatran, thrombolysis, novel oral anticoagulants

P102. Superficial siderosis of the central nervous system due to asymptomatic spinal myxopapillary ependymoma: a case report

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Superficial siderosis of the central nervous system (CNS) is a rare neurological condition characterized by hemosiderin deposition in the subpial layers of the brain and spinal cord, presumably resulting from chronic bleeding into the subarachnoid space.

We report the case of a 61-year-old male with progressive bilateral hearing loss over eight years, who recently developed behavioral changes, gait ataxia, and recurrent falls. Following one such fall, a head trauma led to an emergency CT scan which was unremarkable. Subsequently, the patient developed aggressive behavior, high fever, and obtundation, prompting admission to the infectious disease department. A comprehensive work-up for systemic and CNS infections was performed. Lumbar puncture revealed xanthochromic cerebrospinal fluid with markedly elevated protein levels, normal cytology and glucose, and a negative panel for autoimmune encephalitis.

Brain MRI revealed extensive leptomeningeal hemosiderin staining along bilateral cerebral sulci, cerebellar folia, and the pial surface of the brainstem—findings suggestive of superficial siderosis. A brain CT angiography ruled out major vascular abnormalities. Spine MRI, which also demonstrated whole-spine SWI changes consistent with subpial hemosiderin deposition, identified a large expansive L3–L5 intradural lesion. Surgical resection of this hemorrhagic, friable mass was performed, and histopathology confirmed the diagnosis of myxopapillary ependymoma.

As systemic evaluations were negative for visceral siderosis, we attributed the hemosiderin deposition to chronic, low-grade hemorrhage from the previously asymptomatic spinal myxopapillary ependymoma.

Keywords: superficial siderosis, sensorineural hearing loss, myxopapillary ependymoma

P103. Intra-arterial thrombolysis and mechanical thrombectomy in basilar artery occlusion – a case report

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Basilar artery occlusion is a life-threatening emergency associated with high morbidity and mortality. Prognosis depends on early diagnosis and timely reperfusion therapy.

We report the case of a 51-year-old male patient with no prior medical history who presented with sudden-onset neurological symptoms, including diplopia, acute hearing loss, and right-sided hemiparesis, with an initial NIHSS score of 8. Non-contrast brain CT showed no acute pathology. Cerebral CT angiography revealed right vertebral artery (V4) occlusion and complete basilar artery occlusion. Exploratory angiography also showed subocclusion of the left vertebral artery at its origin.

Intra-arterial thrombolysis was initiated via the right vertebral artery, followed by two mechanical thrombectomy procedures using a stent retriever, resulting in successful basilar artery recanalization 11 hours after symptom onset.

The procedure was particularly challenging due to the large thrombotic burden, which posed a high risk of fragmentation and distal embolization. Additionally, the absence of an aspiration device required the exclusive use of a stent retriever, increasing procedural complexity.

Despite these limitations, the intervention was successful. The patient showed significant clinical improvement, with a reduction in the NIHSS score to 4.

Conclusions. This case underscores the importance of prompt diagnosis and timely endovascular intervention in basilar artery occlusion. The favorable outcome highlights the potential efficacy of intra-arterial thrombolysis and mechanical thrombectomy, even within an extended therapeutic window. It also illustrates the critical role of interdisciplinary collaboration and advanced imaging techniques in the management of posterior circulation ischemic strokes.

Keywords: basilar artery occlusion, intra-arterial thrombolysis, mechanical thrombectomy, posterior circulation stroke, endovascular therapy, stroke intervention

P104. Unveiling atypical stiff-person syndrome – a journey from painful muscle spasms to therapeutic success

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Introduction. Stiff-person syndrome (SPS) is a rare neurological disorder characterized by muscle stiffness and recurrent painful spasms that can worsen over time, often associated with antibodies against glutamic acid decarboxylase (anti-GAD). This case presents an atypical form of SPS in a female patient with latent autoimmune diabetes in adults (LADA).

Methodology. A 52-year-old female with a history of poorly controlled LADA was evaluated for progressive, involuntary, and extremely painful muscle contractions affecting the perineal and gluteal areas, associated with urinary incontinence. Diagnostic workup included spinal MRI, electrophysiological studies, and detection of anti-GAD antibodies in both serum and cerebrospinal fluid (CSF).

Results. The diagnosis of SPS was supported by elevated anti-GAD antibody titers (>2000 in both serum and CSF) and clinical presentation with episodic painful muscle spasms triggered by sudden movement, emotional stress, or tactile stimuli. The patient's symptoms significantly decreased in frequency and severity following plasmapheresis, with almost complete remission observed during hospitalization. A complication of internal jugular vein thrombosis developed during plasmapheresis, requiring anticoagulant therapy. With symptom remission, blood glucose control also improved.

Conclusion. This case demonstrates an atypical presentation of SPS with unusual perineal involvement and intermittent urinary incontinence. Early use of plasmapheresis was effective in reducing symptoms and improving the patient's quality of life. Long-term management includes symptomatic treatment with benzodiazepines and periodic immunoglobulin therapy to prevent recurrence.

Keywords: stiff-person syndrome, LADA, anti-GAD antibodies, plasmapheresis

P105. Severe neurological complications in a case of systemic autoimmune disease: a diagnostic and therapeutic challenge

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Introduction. Rheumatologic autoimmune diseases, such as systemic sclerosis, polymyositis, and rheumatoid arthritis, can cause neurological complications through inflammatory, ischemic, and even infectious mechanisms, particularly as a result of immunosuppressive treatments. The involvement may be central or peripheral, and in some cases, secondary muscle ischemia due to peripheral vascular damage can contribute to the clinical picture, mimicking peripheral nervous system involvement.

Case presentation. A 57-year-old female patient, known for systemic sclerosis (diffuse cutaneous form) overlapping with polymyositis and seropositive rheumatoid arthritis, under treatment with methotrexate and tocilizumab, presented with sudden-onset flaccid paraparesis, hypoesthesia at the D10 level, and acute urinary retention. Cervical and dorsal MRI revealed an epidural spinal abscess from D4 to D8, which was surgically treated with a good postoperative evolution and nearly complete regression of the motor deficit. In this context, immunosuppressive treatment was discontinued.

Four months postoperatively, the patient presented again with intense pain and paresthesia localized distally in the lower limbs. Electroneurography excluded polyneuropathy, and electromyography revealed myopathy with distal denervation elements. An arterial echo-Doppler examination of the lower limbs showed calcinosis and arterial stenosis. A diagnosis of ischemic microangiopathic myopathy was established, and vasodilator treatment with alprostadil was initiated, with a mildly favorable outcome. The patient was referred to the rheumatology department for reevaluation and resumption of specific treatment.

Conclusions. The spinal abscess was a severe neurological complication occurring in the context of autoimmunity and immunosuppression. The discontinuation of immunosuppressive therapy facilitated the development of ischemic microangiopathic myopathy. This case highlights the importance of rigorous monitoring and a multidisciplinary approach in the management of patients with systemic autoimmune diseases.

Keywords: systemic sclerosis, polymyositis, rheumatoid arthritis, spinal abscess, microangiopathic myopathy, immunosuppression

P106. Anti-Hu–associated paraneoplastic neurologic syndrome due to small cell lung cancer: a case report

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Paraneoplastic neurologic syndromes (PNSs) are a large group of rare, immune-mediated disorders that occur as a complication of cancer without direct invasion of the nervous system by malignant cells. PNSs typically manifest prior to the detection of cancer, making early recognition particularly challenging. However, they can less frequently appear in patients with a known malignancy, and in that scenario, they may signal cancer recurrence or reflect an iatrogenic augmented immune response in the setting of immune checkpoint inhibitors.

We present the case of a 65-year-old male with a 4-year history of small cell lung cancer, initially treated with chemotherapy and subsequently switched to immunotherapy (Durvalumab) two years earlier. The patient was considered oncologically stable following remission of hepatic metastases. He developed gait imbalance, vertigo, and paresthesias over four weeks. Neurological examination revealed pancerebellar ataxia, nystagmus, proprioceptive loss, and areflexia. Brain MRI showed only diffuse cerebellar atrophy; CSF analysis was unremarkable. Nerve conduction studies revealed sensory axonal polyneuropathy. A serum onconeuronal antibody panel confirmed high-titer anti-Hu antibodies. A contrast-enhanced thoraco-abdomino-pelvic CT revealed a new suprarenal mass suggestive of metastatic disease, not present at the onset of neurological symptoms. Corticosteroid pulse therapy was initiated, but with no clinical benefits.

We report the rare combination of two paraneoplastic, anti-Hu–associated neurological syndromes – sensory polyneuropathy and cerebellar degeneration – in a patient with longstanding SCLC treated with immunotherapy, in an apparent state of oncologic sta-

bility. The detection of a new suprarenal mass suggests possible tumor reactivation and may represent the trigger for the autoimmune response.

P107. The "superpower" of next-generation sequencing – more than just a diagnosis

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Identifying genomic variations that cause disease has long been a central goal of modern medicine and numerous research initiatives. The rapid development of next-generation sequencing (NGS) technologies has revolutionized our understanding of the human genome, enabling precise diagnoses and appropriate therapeutic decisions in many neurological disorders.

We present the case of a 51-year-old female patient with progressive dystonia and neuromotor deficit, with onset at the age of 39. A clinical suspicion of Parkinson-like disease was raised but not accepted by the patient, who instead pursued alternative treatments, including supplements, vitamins, and amino acids. Her medical history included Hashimoto's thyroiditis, rheumatoid arthritis, gluten and lactose intolerance, and sulfonamide allergy. Following a genetic consultation, whole exome sequencing (WES) was recommended.

WES analysis revealed a homozygous pathogenic missense variant c.758G>A in exon 7 of the PRKN (PARK2) gene, responsible for Parkinson's disease, juvenile type 2. Additionally, two clinically uncertain variants were identified in the ZFAT and SEC23B genes, associated with susceptibility to autoimmune thyroiditis and Cowden syndrome, respectively.

The genetic diagnosis represented a turning point for the patient, shifting her perspective and guiding her toward a better-founded therapeutic approach. This case illustrates the major impact of NGS not only in uncovering a clear genetic etiology but also in reshaping the patient's relationship with the disease, allowing for more informed decisions and improved management.

P108. A case of Creutzfeldt-Jakob disease with negative biomarkers

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Introduction. Several biomarkers help diagnose this condition, although they are not specific—such as 14–3–3 protein, total tau (T-tau), and neuron-specific enolase (NSE).

Method. We admitted a 73-year-old woman who had previously undergone an MRI that showed signal hyperintensity in the bilateral thalamic region and the right temporo-parieto-occipital area (T-P-O) on FLAIR sequences. She presented with persistent vertigo despite medication, along with balance issues.

Results. Neurological examination revealed a slight motor deficit in the right limbs (MRC = 4/5), positive finger-to-nose and heel-to-shin tests on the right side, and an ataxic gait. At times, the patient appeared confused and disoriented. The MMSE score was 20 points. Lumbar puncture revealed 27 cells/mm^3. EEG showed diffuse brain dysfunction and sharp waves. The patient was treated with corticosteroids, with temporary improvement. A prion disease was suspected, and testing for several biomarkers was recommended—all of which returned negative. A SPECT scan was performed, and the results suggested Creutzfeldt-Jakob disease.

Conclusion. Although CSF biomarkers are important diagnostic tools in prion diseases, negative findings – such as in this case – require clinicians to maintain a broad differential diagnosis and rely on clinical, imaging, and electrophysiological evidence.

Keywords: prion disease, Creutzfeldt-Jakob, CSF biomarkers

P109. A rare cause of benign intracranial hypertension secondary to venous stenosis in the context of Eagle syndrome

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Introduction. Idiopathic intracranial hypertension (IIH) is a clinical syndrome characterized by elevated intracranial pressure (ICP) in the absence of a detectable intracranial mass or hydrocephalus. While IIH is traditionally considered idiopathic, venous outflow obstruction may contribute to elevated ICP. Eagle syndrome is a rare condition characterized by elongation of the styloid process, which can compress adjacent neurovascular structures and lead to local changes in venous blood flow.

Materials and methods. We evaluated a 55-year-old female patient for pulsatile tinnitus, progressive visual loss, and headache, with worsening symptoms over the five years preceding admission.

Results. The patient's ophthalmologic examination revealed no abnormalities, including the absence of papilledema. Cerebral magnetic resonance imaging (MRI) demonstrated features suggestive of IIH, including tortuous optic nerves and empty sella. MR venography revealed stenosis of the left transverse sinus, confirmed by digital subtraction angiography. Common causes of prior cerebral venous thrombosis were excluded. CT angiography of the neck vessels revealed an elongated styloid process compatible with Eagle syndrome, with associated jugular compression.

The patient was started on oral acetazolamide, with no significant improvement in her symptoms. Given her debilitating chronic condition, endovascular transverse sinus stenting was performed. At the 3-month follow-up, the patient reported resolution of tinnitus and improvement in visual acuity.

Conclusion. Venous sinus stenosis should be considered in all IIH evaluations, with Eagle syndrome included as a potential contributor to venous outflow impairment.

Keywords: idiopathic intracranial hypertension, sinus stenosis, Eagle syndrome

P110. Complex interplay of spontaneous intramedullary hemorrhage, anticoagulation, ischemic stroke and subdural hematomas in a high-risk patient: therapeutic challenges and outcomes

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Introduction. Spontaneous intramedullary hemorrhage is a rare condition, with most cases associated with specific etiologies such as vascular malformations or tumorous lesions. However, there are reports implicating anticoagulation therapy and hereditary or acquired bleeding disorders as possible causes.

Methods. A 75-year-old female patient with a history of biological mitral valve prosthesis, congestive heart failure (NYHA II), atrial fibrillation treated with apixaban, medullary hypoplasia with associated pancytopenia and splenomegaly, was admitted for sensory impairment, motor weakness of the lower limbs, and bowel and bladder dysfunction with progressive onset in the weeks prior to admission.

Neurological examination revealed spastic paraparesis, severe hypoesthesia below the T10 dermatome, impaired joint position and vibration sense, acute urinary retention, and constipation. Thoracic spinal MRI showed a spinal cord hematoma extending from T5 to T10. Digital subtraction angiography did not identify any vascular malformations.

Results. Anticoagulation therapy was paused for two weeks, without clinical improvement. Upon reinitiation, the patient developed an ischemic stroke in the right middle cerebral artery territory, followed by the appearance of spontaneous bilateral fronto-temporo-parietal subdural hematomas measuring 8–9 mm in thickness. Unfortunately, the outcome was fatal.

Conclusion. This case highlights the delicate balance required in managing anticoagulation in patients with multiple comorbidities. It underscores the importance of early diagnosis and tailored therapeutic strategies to reduce the risk of adverse outcomes.

Despite advancements in the understanding of intramedullary hemorrhage, challenges remain in timely diagnosis and personalized treatment, warranting further research to improve prognosis.

Keywords: spontaneous intramedullary hemorrhage, anticoagulation therapy, subdural hematoma, pancytopenia, ischemic stroke

P111. The overlooked role of Eagle syndrome in carotid artery dissection and stroke

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Introduction. Eagle syndrome is a condition caused by an elongated styloid process or a calcified stylohyoid ligament, which can compress nearby neurovascular structures. The most common clinical presentation of Eagle syndrome is orofacial and cervical pain triggered by neck movements, but in rare instances, it may lead to cervical carotid artery dissection and stroke.

Materials and methods. We present the case of a 54-year-old male who developed right-sided weakness and aphasia during an off-road car race. Initial imaging at a regional hospital revealed no significant brain lesions and left internal carotid artery (ICA) occlusion. Intravenous thrombolysis was administered, and the patient was transferred to our center for endovascular treatment.

Results. The endovascular procedure led to partial ICA recanalization, revealing an extensive dissection from the C1 to C3 segment, as well as a distal middle cerebral artery occlusion. A stent was deployed in the proximal dissection area, and intravenous Cangrelor was initiated. After the procedure, the patient remained neurologically stable, with an NIHSS score of 23. Subsequent computed tomography angiography of the head and neck showed that the dissection site was immediately adjacent to an elongated styloid process in direct contact with the ICA. The patient's clinical condition progressively improved to an NIHSS score of 11 at discharge.

Conclusion. Orofacial and cervical pain not explained by common causes and triggered by neck movements should raise suspicion of Eagle syndrome, which may benefit from surgical treatment. Importantly, the morphology of the styloid process should be routinely evaluated in all patients with carotid artery dissection as a potential underlying cause.

Keywords: Eagle syndrome, elongation of the stylohyoid process, carotid artery dissection, stroke

P112. Neurological manifestations in advanced HIV infection: recurrence of cerebral toxoplasmosis versus onset of neurosyphilis – diagnostic and therapeutic challenges

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Introduction. Human immunodeficiency virus (HIV) infection leads to progressive immunodeficiency, making patients susceptible to various opportunistic infections and neurological complications. Cerebral toxoplasmosis is among the most prevalent opportunistic infections of the central nervous system in individuals with HIV, occurring due to the reactivation of latent Toxoplasma gondii infection in patients whose CD4 lymphocyte counts are below 100/mm^3. A concurrent infection with Treponema pallidum may further complicate the clinical scenario.

Case presentation. We present the case of a young patient with a history of HIV infection and cerebral toxoplasmosis, admitted to the emergency department for motor aphasia and right orofacial focal motor seizures. Brain imaging (CT, MRI) revealed multiple calcified lesions with vasogenic edema, and the electroencephalogram indicated left temporal irritative abnormalities. His medical history includes a severe episode of neurological impairment in 2022, characterized by flaccid tetraparesis, dysarthria, cognitive decline, and multiple infectious complications. Following specific treatment (antibiotics, antifungals, and antiretrovirals), the subsequent evolution was favorable. The current investigation reveals a positive TPLA test, suggesting a previously undiagnosed case of neurosyphilis.

Conclusions. This case highlights the complexity of neurological manifestations in advanced HIV infection, where opportunistic coinfection can lead to recurrent or progressive symptoms. Diagnostic evaluation should encompass both toxoplasmosis reactivation and syphilis coinfection, as well as syphilitic cerebral

vasculopathy. A multidisciplinary approach is essential for optimal long-term management. Therapeutic management is complicated by the presence of epileptic seizures, which can be triggered by standard penicillin therapy. Alternative options include ceftriaxone or doxycycline, depending on tolerance and patient profile.

Keywords: HIV, toxoplasmosis, neurosyphilis, motor aphasia, seizures

P113. AMSAN associated with an inflammatory encephalopathy of uncertain etiology – challenges in the management of a complex neurological case

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Autoimmune neurological diseases represent a broad spectrum of conditions in which the immune system targets structures of the central or peripheral nervous system. Immune involvement of both the central and peripheral nervous systems simultaneously is a rare situation, but one with significant clinical relevance. This association suggests a systemic immune process with widespread tropism, possibly mediated by common autoimmune mechanisms or by cross-reaction between central and peripheral antigens.

These conditions can have an acute or insidious onset, with variable clinical manifestations, ranging from behavioral and consciousness disturbances to severe motor or sensory deficits. In the case of the acute onset of an autoimmune disease involving both the central and peripheral nervous systems, differential diagnosis becomes essential, given that the symptoms may mimic other serious conditions such as intoxications, infections, paraneoplastic syndromes, or metabolic disorders.

We present the case of a 63-year-old patient who sought emergency care presenting with flaccid tetraparesis, oculomotor disturbances, disorientation, and phenomena of acute psychosis. This case highlights the importance of an integrated and dynamic approach to a mixed neurological presentation, where early therapeutic intervention can decisively influence functional prognosis.

Keywords: autoimmune neurological diseases, simultaneous involvement of CNS and PNS

P114. Early diagnosis and management of von Hippel–Lindau disease: a case report of spinal and cerebellar hemangioblastomas with neurological involvement

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Introduction. Von Hippel–Lindau (VHL) disease is a hereditary condition characterized by the development of multiple tumors, both benign and malignant, in various organs. Hemangioblastomas, particularly in the central nervous system, are a hallmark of VHL disease. These tumors are commonly misdiagnosed as hemangiomas due to their similar appearance. Progressive neurological symptoms, such as ataxia and spastic paraparesis, often accompany the disease, making early diagnosis and management critical. This case highlights the importance of considering VHL disease in patients with hemangiomas or hemangioblastomas, particularly when clinical features suggest neurological involvement.

Case report. A 54-year-old woman presented with spinal and cerebellar hemangioblastomas, initially diagnosed as hemangiomas due to their benign appearance. However, as the tumors increased in size, clinical suspicion of VHL disease was raised. Genetic testing and biopsy of the cerebellar hemangioblastomas confirmed the diagnosis of VHL disease. The patient developed progressive ataxia and spastic paraparesis, both of which significantly impacted her quality of life. Additionally, a pancreatic cyst was noted, which is another potential manifestation of VHL disease. The progression of neurological symptoms prompted a multidisciplinary approach to manage the patient's condition.

Conclusion. This case underscores the importance of considering VHL disease in patients with multiple hemangioblastomas, especially when accompanied by neurological symptoms. Early diagnosis through genetic testing and biopsy, along with careful monitoring, is crucial for guiding treatment and managing the disease. VHL disease presents with a variety of manifestations, and a thorough evaluation is essential for proper diagnosis and long-term management. Surveillance for associated tumors and neurological deficits is necessary to optimize patient care.

P115. Pseudomedian nerve palsy: a mask for ischemic stroke

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Pseudomedian nerve palsy is a rare pathology characterized by isolated monoparesis in the median nerve territory, secondary to ischemic injury of the central nervous system. There are no focal neurological signs, the etiology is atherothrombotic, and the differential diagnosis includes carpal tunnel syndrome, anterior interosseous syndrome (Kiloh-Nevin), and pronator syndrome. Pseudoparesis of the ulnar or radial nerves is more common.

We present the case of an 83-year-old hypertensive, dyslipidemic male who developed muscle weakness in the first and second fingers of his right hand, with total inability of flexion, ten days prior to presentation. The symptoms began during the day, without cervical or right upper limb pain, local trauma, or sensory disturbances. Neurologically, the patient presented with an impossible right medio-median grip, a deficit in flexion of the thumb and index finger of the right hand, and inability to oppose the thumb, without other neurological signs.

We initially suspected median nerve neuropathy, and the investigative work-up included a biological profile for neuropathies, electrodiagnostic evaluation (nerve conduction studies and electromyography), and radio-cubital-carpal joint ultrasound—all within normal limits.

Subsequently, raising suspicion of a central cause, we performed brain MRI, which revealed a subacute infarction of the left precentral gyrus. The ischemic stroke (IS) screening included Doppler ultrasound of the cervical-cerebral vessels, which detected a tight stenosis of the cavernous-clinoid segment of the left internal carotid artery (ICA), and CT angiography, which revealed two tight stenoses at the cavernous-clinoid and clinoid-ophthalmic junctions of the left ICA.

Secondary stroke prevention was initiated, with favorable evolution, including the reappearance of the medio-median grip after two weeks.

This pathology mimics median nerve injury and demonstrates how a peripheral nerve condition can sometimes reveal an underlying IS, highlighting the importance of early diagnosis for effective treatment and prognosis.

P116. Seronegative limbic encephalitis: diagnostic uncertainty and therapeutic challenges in a case report

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Introduction. Limbic encephalitis is a form of autoimmune encephalitis characterized by neuropsychiatric symptoms, working memory deficits, seizures, and focal neurological signs. It involves inflammation of the limbic system, and while specific neuronal antibodies are commonly found, seronegative cases have been reported. Additionally, it may precede the detection of an underlying neoplasm.

Methods. We report the case of a 50-year-old female patient who presented to the emergency department with anterograde amnesia and behavioral disturbances, with symptom onset approximately three days prior to evaluation.

Results. A lumbar puncture was performed, revealing cerebrospinal fluid (CSF) alterations characterized by pleocytosis (31 cells/µL), a mild increase in protein concentration, and the presence of the 14-3-3 protein. Testing for autoimmune neuronal surface (NMDA-R, CASPR2, LGI1, GABA-BR, mGluR5, AMPAR, GAD) and intracellular antibodies was negative, as was the BioFire meningitis/encephalitis panel. Comprehensive serological analysis, including tumor markers and infectious agents, yielded negative results. MRI identified bilateral T2 hyperintensities within the medial temporal lobes. Serial electroencephalograms documented predominantly right-sided frontotemporal epileptiform discharges. A thoraco-abdomino-pelvic CT scan revealed no evidence of neoplastic lesions. The diagnosis of limbic encephalitis was corroborated by both the clinical presentation and MRI findings. The patient was initiated on intravenous immunoglobulin, methylprednisolone, antiepileptic therapy, and antipsychotics, with subsequent clinical improvement, reinforcing the autoimmune etiology of the condition.

Conclusions. We present a typical case of limbic encephalitis; however, the uniqueness of this case lies in its seronegative nature and the absence of an identified neoplasm.

Keywords: limbic encephalitis, seronegative, anterograde amnesia

P117. Memory dysfunction and temporal lobe damage following HSV-1 encephalitis: exploring an autoimmune etiology

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Introduction. Herpes simplex encephalitis (HSE) is one of the most common forms of sporadic viral encephalitis, typically presenting with a monophasic course characterized by rapid onset and brief duration. However, persistent neurological deficits, particularly cognitive impairment, may develop despite appropriate antiviral therapy.

Methods and results. We report the case of a 30-year-old male diagnosed with herpes simplex virus type 1 (HSV-1) encephalitis, who was treated with standard antiviral therapy. During the acute phase, he experienced a generalized epileptic seizure, requiring antiepileptic treatment with Levetiracetam, later transitioned to Lamotrigine due to continued epileptiform activity on electroencephalogram (EEG).

Despite clinical stabilization, follow-up revealed significant cognitive deficits. The patient demonstrated profound retrograde and anterograde amnesia, with a Mini-Mental State Examination (MMSE) score of 26/30 and a Montreal Cognitive Assessment (MoCA) score of 21/30—findings notable given his high educational background (18 years).

Brain MRI, both native and contrast-enhanced, showed extensive gliotic changes predominantly in the bilateral temporal and insular cortices, with additional involvement of the right orbito-basal frontal cortex—regions associated with memory and executive function. These structural abnormalities correlated with the patient's neurocognitive symptoms.

Given the persistence and progression of symptoms, serological testing for autoimmune markers, including anti-GAD and anti-NMDA receptor antibodies, was performed to assess for autoimmune encephalitis following HSE (AEaHSE).

Conclusions. This case underscores the potential for long-term cognitive impairment and structural brain damage following HSV-1 encephalitis. The correlation between MRI findings and neurocognitive dysfunction suggests a possible autoimmune mechanism, reinforcing the importance of early recognition and management of secondary autoimmune processes to improve long-term outcomes.

Keywords: herpes simplex, encephalitis, autoimmune

P118. From disease progression to functional improvement: siponimod's therapeutic impact in secondary progressive multiple sclerosis

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Introduction. Multiple sclerosis (MS) is a chronic neuroinflammatory and neurodegenerative disorder of the central nervous system. Treatment requires personalized disease-modifying therapies (DMTs) to slow disability progression, particularly in secondary progressive MS (SPMS), where shifting disease mechanisms demand tailored therapeutic approaches.

Methods. We conducted a longitudinal clinical and imaging evaluation of this case, including regular neurological assessments (Expanded Disability Status Scale [EDSS] scoring), annual brain and spinal cord MRI with gadolinium contrast, and monitoring of treatment adherence and adverse effects. The decision to switch therapies was made through multidisciplinary consensus based on clinical progression, MRI activity, and current guidelines for SPMS management.

Case presentation. We present the case of a 47-year-old patient diagnosed with relapsing-remitting multiple sclerosis (RRMS) four years ago, initially treated with first-line interferon beta-1a for three years. Over the past year, the disease course indicated a transition to secondary progressive MS (SPMS), with a gradual worsening of disability reflected by an increase in the EDSS score. Following evaluation by the therapeutic platform, the treatment was switched to siponimod, a selective sphingosine-1-phosphate (S1P) receptor modulator. After initiating the new therapy, the patient showed significant clinical improvement, with a reduced EDSS score corresponding to partial re-

covery of mobility and functional stability. MRI monitoring confirmed the absence of new active lesions, and the treatment was well tolerated, with no significant adverse effects. This case highlights the importance of adapting therapeutic strategies in SPMS and the role of personalized treatment in improving prognosis and quality of life for patients.

Conclusions. This case highlights siponimod's potential to reverse disability in SPMS after first-line therapy failure, supporting its role in personalized DMT escalation strategies.

Keywords: multiple sclerosis, siponimod, treatment optimization, disease-modifying therapies, EDSS

P119. A stroke of misfortune: severe ischemic stroke in a young woman with suspected antiphospholipid syndrome

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Introduction. Ischemic stroke in young adults remains an uncommon yet critical presentation, often signifying an underlying prothrombotic state. Among these, antiphospholipid syndrome (APS) stands out as a potential cause of life-threatening arterial thromboses.

Methods. We reviewed the clinical, radiological, and laboratory data of a patient presenting with severe acute ischemic stroke, including neuroimaging, laboratory testing, cardiac evaluation, and autoimmune workup.

Results. We describe the case of a 25-year-old woman who arrived at our stroke center five hours after symptom onset, having received intravenous recombinant tissue plasminogen activator (rTPA) at another regional center. Despite an initial ASPECTS of zero, urgent mechanical thrombectomy achieved complete recanalization. The patient subsequently developed large hemispheric edema necessitating prompt decompressive craniectomy, which contributed to a favorable neurological course. Notably, on follow-up imaging at approximately one month, she exhibited significant cortical reperfusion across the entire right cerebral hemisphere. Laboratory investigations revealed mild thrombocytopenia, anemia, prolonged aPTT, and a positive lupus anticoagulant assay, raising suspicion for APS, pending confirmatory testing in 12 weeks according to the Sapporo criteria.

Conclusion. This case emphasizes the potential for aggressive endovascular treatment and early surgical intervention to dramatically improve outcomes, even when initial imaging suggests a poor prognosis. Timely recanalization strategies and decompressive surgery may enhance both functional recovery and long-term quality of life for patients with severe ischemic strokes. Furthermore, this case highlights the necessity of an extensive etiological evaluation in young individuals, where APS can manifest unexpectedly.

Keywords: stroke, antiphospholipid syndrome

P120. A sticky situation: recurrent in-stent stenosis in a woman with elevated lipoprotein(a)

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Introduction. Recurrent in-stent stenosis remains a significant clinical challenge, particularly in patients without traditional cardiovascular risk factors. Lipoprotein(a) [Lp(a)] has emerged as an independent and genetically determined risk factor for atherosclerotic cardiovascular disease and may contribute to neointimal hyperplasia and stent failure.

Methods. We retrospectively reviewed the clinical, imaging, and laboratory data of a patient presenting with recurrent episodes of in-stent stenosis despite optimal medical therapy. Standard cardiovascular risk assessment was performed alongside advanced lipid profiling, including measurement of lipoprotein(a) levels. Interventional strategies, medical management, and follow-up outcomes were documented to assess progression and therapeutic response.

Results. A 48-year-old woman, with a history of dyslipidemia and no other significant cardiovascular risk factors, underwent carotid artery stenting for symptomatic 70% right internal carotid artery (ICA) stenosis. There were no periprocedural complications, and the patient was discharged with clopidogrel and aspirin. Despite optimal medical therapy, good adherence, and a favorable lipid profile, follow-up imaging at 10 months revealed 40–50% intrastent restenosis. At one year, the restenosis progressed to 70%, prompting a switch from clopidogrel to ticagrelor in combination with aspirin. Balloon angioplasty was performed, but the stenosis continued to progress on subsequent imaging. Further evaluation revealed lipoprotein(a) levels more than twice the upper limit

of normal. The patient is now assessed for eligibility for emerging lipoprotein(a)-lowering therapies.

Conclusion. This case underscores the potential role of elevated lipoprotein(a) in recurrent in-stent restenosis, even in the absence of other major risk factors. Recognition of Lp(a) as a contributing factor is essential for risk stratification and may inform future therapeutic strategies in patients with unexplained restenosis.

Keywords: in-stent restenosis, lipoprotein(a), angioplasty, neointimal hyperplasia

P121. Neurolymphomatosis in a chemotherapy-treated non-Hodgkin's lymphoma patient: a case of cranial nerve palsy and quadriparesis

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Introduction. Neurolymphomatosis is a rare complication of lymphoma infiltration of cranial or peripheral nerves, more commonly within B-cell non-Hodgkin's lymphoma. Clinical presentation varies, causing multiple or isolated cranial nerve palsies. It may be the initial manifestation (primary neurolymphomatosis), or it may occur during disease progression or relapses (secondary neurolymphomatosis).

Methods. Clinical case of a 49-year-old patient, known with chemotherapy-treated mantle-cell non-Hodgkin's lymphoma, admitted for multiple, saltatory cranial nerve palsies and quadriparesis. Symptoms worsened progressively over approximately one month, also being associated with deterioration of the general condition.

Results. At admission, the patient was obtunded. Neurological examination revealed absent pupillary light reflexes, right eye exophthalmos with minimal abduction capacity preserved, dysphonia, and dysphagia. He also reported numbness in the left trigeminal nerve territory and lumbago, and presented with quadriparesis, predominantly in the lower limbs (paraparesis). Brain CT described multiple contrast-enhancing, homogeneous, extraneural space-occupying lesions. Native and post-contrast brain MRI showed nodular thickening and contrast enhancement of the optic chiasm and optic tracts bilaterally, the left olfactory and trigeminal nerves, and linear thickening of the facial and vestibulocochlear nerves bilaterally. MRI also revealed an expansive process located in the right orbit, with secondary mass effect. Imaging assessment was completed with cervico-thoraco-lumbar spine MRI, which described enhancement and spinal stenosis at the T12–L5 level, secondary to leptomeningeal infiltration.

Conclusion. Neurolymphomatosis should be considered in cranial nerve palsies, sometimes being the first manifestation in an undiagnosed lymphoma. After ruling out other potential causes (chemotherapy-induced neuropathy, paraneoplastic syndromes), neurolymphomatosis was confirmed based on MRI findings showing cranial nerve lymphoma infiltration and secondary spinal cord involvement.

Keywords: cranial nerve palsy, lymphoma

P122. Longitudinally extensive transverse myelitis with monophasic evolution – a case study

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Introduction. Longitudinally extensive transverse myelitis (LETM) has multiple etiologies, including CNS demyelinating disorders, systemic autoimmune/inflammatory diseases (SADs), infections, and paraneoplastic syndromes. In 15–30% of the cases, the cause remains unknown.

Method. We present the case of a 59-year-old female with monophasic LETM.

Results. The patient developed sudden severe paraparesis, without any trigger or preceding symptoms (e.g., pain), and no recovery afterwards. Neurological examination revealed spastic asymmetrical paraparesis, severe hypoesthesia below T4 in all sensory modalities, and urinary/fecal incontinence. The upper limbs and cranial nerves were unaffected. Spine MRI identified a T2–T8 transverse lesion with hyperintense signal on T2/STIR sequences, patchy contrast enhancement, and restricted diffusion, without extramedullary intradural flow voids. CSF analysis revealed mild lymphocytic pleocytosis and elevated protein concentration, with normal glucose levels. Although she did

not have a spinal artery syndrome, the diagnosis of spinal arterial infarct was considered, and she was prescribed antiplatelet therapy.

After a year and a half, we were asked for a second opinion and retrospectively considered the diagnosis of LETM, as severe inflammatory lesions can show restricted diffusion. The biological workup for demyelinating disorders (anti-MOG and AQP4 antibodies), neoplasms, and neuroinfections was negative, except for positive anti-HCV antibodies. Considering the monophasic course, we did not initiate immunosuppressive or immunomodulatory therapy.

Conclusion. Hepatitis C virus primarily affects the peripheral nervous system, but rare cases of CNS involvement have been reported, likely due to autoimmune mechanisms. Although LETM could be linked to HCV, other unidentified viral triggers or spinal ADEM cannot be excluded.

P123. Cerebral amyloid angiopathy presenting as hemorrhagic stroke with transient loss of consciousness and focal neurological deficits: a case report

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Cerebral amyloid angiopathy (CAA) is a small vessel disease characterized by β-amyloid deposition in the walls of leptomeningeal and cortical arteries, predisposing patients to lobar hemorrhages and cognitive impairment. It is a major cause of non-hypertensive intracerebral hemorrhage in elderly patients and is associated with microbleeds, as seen on susceptibility-weighted imaging (SWI), fulfilling the Boston criteria.

We present the case of a 55-year-old patient who experienced a single episode of loss of consciousness lasting approximately 30 minutes. Upon regaining consciousness, the patient exhibited language disturbances, agitation, and a left upper limb motor deficit. A computed tomography (CT) scan revealed a right temporo-occipital hemorrhagic stroke. Subsequent MRI with SWI demonstrated multiple microbleeds, confirming the diagnosis of probable CAA according to the Boston criteria, after exclusion of alternative etiologies.

This case underscores the importance of recognizing CAA as a potential cause of spontaneous lobar hemorrhage and transient neurological symptoms. Given the risk of recurrent hemorrhages and cognitive decline, accurate diagnosis through advanced imaging techniques is crucial for guiding management strategies and secondary prevention.

Keywords: cerebral amyloid angiopathy, hemorrhagic stroke, Boston criteria, microbleeds, lobar hemorrhage

P124. Racing against the clock – multiple cardioembolic stroke related to mitral valve fibroelastoma in a 41-year-old pregnant woman

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Introduction. Stroke during pregnancy is a rare encounter, and atypical cardiogenic etiologies, like valvular fibroelastoma, are exceptional. Multifocal presentation, with multiple ischemic lesions in both anterior and posterior cerebral vascular territories, related to subacute endocarditis superimposed on a mitral valve tumor in a pregnant patient, presents a difficult diagnostic approach and challenging treatment options.

Methods. We present the case of a 41-year-old female patient, 26 weeks pregnant and with diabetes mellitus, who presented in our clinic with acute mental status impairment and triparesis. Cerebral magnetic resonance imaging revealed extensive bihemispheric and cerebellar lesions, all with restricted diffusion, suggestive of an embolic source. Shortly afterward, during transesophageal echocardiography, a 12 × 11 mm irregular mass attached to the mitral valve was discovered. Imposed by the continuous cardioembolic risk, open-heart surgery with valvular metallic prosthesis was chosen. This procedure was only possible in an emergency setting, after a safe cesarean delivery of her newborn baby. Both anticoagulation and empiric – and later targeted – antibiotic therapy were initiated, with the histopathologic exam confirming the diagnosis of papillary fibroelastoma with superimposed subacute endocarditis. Despite these challenges, both our patient and her baby managed to surpass all expectations and were finally discharged home.

Conclusions. Rare on their own, stroke during pregnancy and cardiac tumoral pathology were asso-

ciated in our case. Fast recognition and confirmation of a potentially cardioembolic source of stroke was mandatory for both diagnostic and treatment approaches. Fortunately, a multidisciplinary team was assembled, with every link contributing to the favorable outcome and recovery of our patient.

P125. Parkinson's disease and myasthenia gravis – a rare association

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Parkinson's disease is a complex progressive neurodegenerative disease associated with deposition of aggregated alpha-synuclein. Several studies have shown a possible contributing role of immunity in the pathogenesis of Parkinson's disease, some even raising the question of it being the result of an autoimmune process. An association with other autoimmune diseases has been documented.

Myasthenia gravis is an autoimmune neuromuscular junction disorder characterized by fluctuating motor weakness caused by an antibody-mediated immunologic attack against acetylcholine receptors or receptor-associated proteins. To date, fewer than 100 cases of comorbid Parkinson's disease and myasthenia gravis have been reported.

We report the case of a 79-year-old male diagnosed with Parkinson's disease who presented to our clinic with bilateral ptosis for two months. The patient's history revealed a similar episode of unilateral ptosis that had been treated with corticosteroids and an oral cholinesterase inhibitor 18 years prior to admission. Neurological examination revealed a prominent shuffling gait with frequent freezing, asymmetric generalized bradykinesia, and severe bilateral ptosis. Repetitive nerve stimulation showed decrement; antibodies against acetylcholine receptor were strongly positive, and a therapeutic trial with oral pyridostigmine significantly improved the patient's gait and ptosis. Hence, we made the diagnosis of myasthenia gravis.

Initially, the patient's symptoms were well controlled with oral pyridostigmine and azathioprine, but he later developed worsening freezing of gait, which responded well to increasing the levodopa/carbidopa dosage and adding a COMT inhibitor.

Thus, we report a case of myasthenia gravis comorbid with Parkinson's disease, highlighting the possibility of a common autoimmune pathophysiological mechanism.

Keywords: Parkinson's disease, myasthenia gravis, autoimmune, levodopa-responsive, overlap

P126. Novel VCP mutation associated with motor neuron disease?

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Amyotrophic lateral sclerosis is an incurable neurodegenerative disease that affects both upper and lower motor neurons. Aside from ALS, the term motor neuron disease (MND) encompasses other related degenerative conditions affecting motor neurons, such as progressive muscular atrophy—a form of MND characterized by isolated lower motor neuron dysfunction. Over 50 genetic mutations have been linked to ALS.

VCP is a protein encoded by the VCP gene, involved in the clearance of proteins. Mutations in VCP lead to dysregulation of protein homeostasis, resulting in protein aggregation and accumulation, especially of TDP-43 and tau proteins. Cases of both familial and sporadic ALS have been described in association with VCP mutations.

We present the case of a 23-year-old male admitted to our clinic for muscle weakness and fasciculations in all four limbs for 18 months. Neurological examination revealed asymmetric quadriparesis, diminished deep tendon reflexes, generalized muscle atrophy, and spontaneous fasciculations in the muscles of the proximal lower limbs. Electrodiagnostic studies showed neurogenic changes and spontaneous activity in two territories. We made the diagnosis of progressive muscular atrophy. The patient underwent whole genome sequencing, which identified a variant of unknown significance (c.2138G>A) in the VCP gene.

We report a potentially novel mutation in the VCP gene linked to motor neuron disease, underscoring

the importance of genetic testing in these patients. This approach could enhance our understanding of the disease's pathogenesis and pave the way for the development of new targeted therapies.

Keywords: motor neuron disease, genetic testing, progressive muscular atrophy, VCP, VUS

P127. The challenges of diagnosing primary angiitis of the central nervous system – a case report

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Introduction. Primary angiitis of the central nervous system (PACNS) is a rare inflammatory disorder that affects small and medium-sized arteries in the brain and spinal cord. The difficulties in diagnosing PACNS arise from the complex clinical and paraclinical features, which overlap with numerous neurological pathologies.

Material and methods. A 51-year-old patient presented with gait disturbance that began one month prior and progressively worsened, associated with paresthesia in the lower limbs and progressive urinary retention. The neurological exam revealed left homonymous hemianopia, right central facial palsy, spastic paraparesis, bilateral distal paresthesia in the lower limbs, dysarthria, and urinary retention. The etiological workup excluded systemic vasculitis, infectious, or neoplastic pathology.

Cerebrospinal fluid analysis revealed proteinorachia, mononuclear pleocytosis, and significant disruption of the blood–brain barrier. Imaging revealed gliotic lesions in the right occipital, bilateral temporal, left high parietal, and right thalamic regions, with hemosiderin inclusions in the right occipital area, as well as a subacute vascular lesion in the right high frontal area and in the left cerebellar hemisphere, without changes in brain angiography (CT or MRI). PACNS was suspected, and a brain biopsy was recommended but refused by the patient. Antithrombotic treatment combined with corticosteroid therapy was initiated, leading to an improvement in the patient's symptoms.

Conclusions. This clinical case highlights the challenges posed by the diagnosis of PACNS and the importance of correlating the clinical presentation with imaging and cerebrospinal fluid analysis, especially in the absence of a brain biopsy.

Keywords: primary angiitis, cerebral vasculitis, central nervous system

P128. HIV nonatherosclerotic vasculopathy in the era of highly active antiretroviral therapy – a case report

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Introduction. Vasculopathy is a known uncommon complication of chronic HIV infection that may be further exacerbated by highly active antiretroviral therapy (HAART). The aim of this report is to present the complex case of a patient in whom HIV-associated nonatherosclerotic vasculopathy was suspected.

Methods. A 36-year-old female patient presented in our clinic with speech disturbance and right-side hemiparesis. She was known with a history of chronic HIV infection (since 1995), remissive non-Hodgkin lymphoma (since 2005), and was recently diagnosed with mesangial proliferative glomerulonephritis with nephrotic syndrome and chronic kidney disease, as well as renal secondary arterial hypertension and hypercholesterolemia. She was compliant to her regimen of bictegravir/emtricitabine/tenofovir alafenamide plus doravirine, had negative viremia for several years, and followed oral corticosteroids for the renal pathology.

Results. Initial cerebral and vessel imaging suggested acute ischemic stroke with left middle cerebral artery distribution, as well as left internal carotid artery pre-occlusive stenosis in the distal segments. Doppler ultrasonography of the cervical arteries showed no atherosclerotic plaques. Magnetic resonance imaging studies confirmed the diagnosis of stroke and revealed gadolinium enhancement of the arterial wall at the level of the stenosis. Thus, a diagnosis of inflammatory vasculopathy was established and immunosuppressive therapy was continued alongside antiplatelets, statin, and antihypertensives.

Conclusions. Although uncommon, inflammatory vasculopathy is a serious complication of chronic HIV infection and antiretroviral therapy that must be suspected in patients with no other signs of atherosclerosis despite the concurrence of traditional vascular risk factors.

Keywords: HIV nonatherosclerotic vasculopathy, inflammatory, highly active antiretroviral therapy (HAART), pre-occlusive stenosis, gadolinium enhancement, ischemic stroke, corticosteroids, immunosuppressive therapy

P129. Genetic or inflammatory? The challenges of diagnosing cerebral vasculopathies in young adults – a case report

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Introduction. Central nervous system (CNS) vasculopathies represent a broad group of disorders with multiple etiologies, both inflammatory (systemic vasculitis affecting the CNS and isolated CNS angiitis) and non-inflammatory (atherosclerotic disease, autosomal dominant cerebral arteriopathy with subcortical infarcts and leukoencephalopathy — CADASIL, Susac syndrome, Moyamoya disease, fibromuscular dysplasia, reversible cerebral vasoconstriction syndrome, or mitochondrial diseases).

Material and methods. We present the case of a 39-year-old patient with no significant past medical history but with a family history of strokes and extensive colored tattoos on both arms and torso, who presented with mixed transcortical aphasia of sudden onset. The etiological workup excluded infectious pathologies, systemic vasculitis, Fabry disease, multiple sclerosis, sarcoidosis, and autoimmune encephalitis. Imaging studies were highly suggestive of CADASIL — microangiopathic periventricular leukoencephalopathy, vascular lesions in the subcortical white matter in the temporal poles, multiple, focal or confluent. Cerebrospinal fluid analysis excluded inflammatory pathology. Genetic testing for CADASIL syndrome type 1 — NOTCH3 gene — was negative. The patient refused brain biopsy. Correlating the clinical picture, imaging results, and genetic testing, the following diagnoses were considered: central nervous system vasculitis, mitochondrial disease, or inflammatory CADASIL. Corticosteroid therapy was initiated, resulting in remission of aphasia, but with residual bradylalia and bradyphrenia. Complete genome sequencing was recommended.

Conclusions. This clinical case highlights the challenges and limitations of diagnosing cerebral vasculopathy in a young patient. Genetic testing is an important integral part of diagnosing rare neurological diseases.

Keywords: cerebral vasculopathies, genetic testing, young stroke

P130. Trigeminal neuralgia due to a lateral venous sinus aneurysm: a rare condition – case report

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Introduction. Trigeminal neuralgia caused by a right lateral venous sinus aneurysm is a rare but potentially severe condition that poses diagnostic and therapeutic challenges. We report the case of a patient whose persistent headache led to the discovery of a right lateral venous sinus aneurysm, confirmed through neurovascular imaging.

Material and methods. We present the case of a 45-year-old female patient with no significant medical history, who presented to the Neurology Clinic in Sibiu with right-sided pulsatile hemicrania, exacerbated by coughing and sneezing. The symptoms began one year ago but progressively worsened over time in both duration and intensity, accompanied by otalgia with pain radiating to the mandible and maxilla. One month before seeing a neurologist, she experienced daily severe headaches (10/10 on the pain scale) with dizziness and ear pain. Neurological examination was normal. Imaging (CT angiography and MR angiography) revealed a right lateral sinus aneurysm. The aneurysm was treated with coiling and stenting, leading to reduced blood flow without complications.

Conclusions. Trigeminal neuralgia caused by a right lateral venous sinus aneurysm is uncommon but important, requiring clinical suspicion in atypical or treatment-resistant cases. Early imaging, such as MR or CT angiography, is key for diagnosis, and endovascular embolization offers effective symptom relief and prevents complications. This case emphasizes the need for careful diagnosis and the role of interventional treatment in managing cerebral vascular issues.

Keywords: secondary headache, venous sinus aneurysm, MR angiography, CT angiography, endovascular intervention

P131. The unreachable enemy: a rare case of cavernous sinus metastasis from head and neck squamous cell carcinoma

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Introduction. Brain metastases are the most common secondary tumors, affecting about 20% of cancer patients. However, cerebral metastases from head and neck squamous cell carcinoma (HNSCC) are uncommon, as these tumors tend to be locally aggressive rather than spreading distantly. Metastases to the cavernous sinus are exceptionally rare, with only a few cases reported.

Materials and methods. We present the case of a 59-year-old male diagnosed with squamous cell carcinoma of the tongue, undergoing radiotherapy and chemotherapy, with post-radiotherapy left jugular vein thrombosis. He presented with left hemicrania and progressive left eye symptoms over recent weeks, including palpebral ptosis, exophthalmos, ophthalmoplegia, epiphora, and superficial hypoesthesia in the left ophthalmic trigeminal territory.

Contrast-enhanced brain MRI revealed an isointense T1, T2, FLAIR expansive mass with homogeneous contrast enhancement, measuring 23 × 17 × 22 mm, located in the left cavernous sinus and encompassing all neurovascular structures at this level.

The patient underwent neurosurgical intervention through a left pterional approach, achieving more than 70% tumor resection and decompression of the oculomotor, trochlear, and maxillary branches of the trigeminal nerve. Histopathological examination confirmed the diagnosis of metastatic squamous cell carcinoma.

Despite surgical and oncological management, the prognosis remained poor. At one month postoperatively, bilateral recurrence in the cavernous sinus and secondary pulmonary, mediastinal, and esophageal lesions were identified.

Conclusions. Although rare, metastases to the cavernous sinus can occur in aggressive primary tumors such as squamous cell carcinoma. Treatment options are limited due to the deep location and involvement of multiple neurovascular structures. Despite aggressive interventions, prognosis remains poor, with palliation often being the only viable option in advanced cases.

Keywords: cavernous sinus, metastasis, head and neck squamous cell carcinoma, multidisciplinary approach

P132. Ischemic stroke in POEMS syndrome: a rare presentation of quasi-moyamoya vasculopathy

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Introduction. POEMS (polyneuropathy, organomegaly, endocrinopathy, monoclonal protein, and skin changes) syndrome is a rare paraneoplastic disorder associated with plasma cell dyscrasia and elevated vascular endothelial growth factor (VEGF). It primarily manifests as severe demyelinating polyneuropathy. While vascular complications are recognized, the association with quasi-moyamoya vasculopathy – marked by intracranial arterial stenosis and collateral network formation – remains exceptionally rare.

Materials and methods. We describe a 51-year-old woman presenting with a one-year history of progressive distal paresthesia and muscle weakness. Evaluation revealed demyelinating polyneuropathy. She was subsequently diagnosed with POEMS syndrome, exhibiting typical features including monoclonal lambda gammopathy, hepato-splenomegaly, hypothyroidism, skin hyperpigmentation, thrombocytosis, thrombotic diathesis, elevated VEGF levels, and a plasmacytoma. Treatment was initiated with chemotherapy (lenalidomide and dexamethasone) and radiation therapy for the pelvic plasmacytoma.

Four months into treatment, the patient developed sudden left hemiparesis. Imaging revealed a right internal watershed ischemic stroke. Digital subtraction angiography confirmed quasi-moyamoya vasculopathy (Suzuki stage 1), with multifocal stenosis of the right internal carotid artery and proximal middle and anterior cerebral arteries, as well as leptomeningeal collateral formation.

Conclusions. This case highlights a rare and complex presentation of POEMS syndrome complicated by quasi-moyamoya vasculopathy, likely mediated by VEGF-induced vascular remodeling. The association underscores the potential for atypical vascular manifestations in POEMS syndrome and the importance of considering rare etiologies in stroke patients with this underlying disorder.

Keywords: polyneuropathy, POEMS syndrome, ischemic stroke, quasi-moyamoya vasculopathy, paraneoplastic syndromes

P133. A particular case of non-aneurysmal subarachnoid hemorrhage in a young patient without risk factors

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Introduction. Non-aneurysmal (idiopathic) subarachnoid hemorrhage accounts for 15–20% of all SAH cases, most commonly (66%) located in the perimesencephalic cisterns. The fatality rate of this vascular event is extremely high – up to 50% – with 1 in 8 people dying outside of the hospital. It shows a slight male predominance and is more prevalent in younger individuals.

Method and results. A 43-year-old male presented with a thunderclap headache, vomiting, and signs of meningeal irritation, all of which started abruptly after a sneeze.

The patient had no prior medical history and was not on any medication. Clinically, there were no cranial nerve deficits, only classical symptoms of meningeal irritation such as nuchal rigidity, moderate to severe headache, photophobia, and a positive Brudzinski sign, all corresponding to a Hunt & Hess score of 1 point. Under antihypertensive treatment, the symptoms resolved completely. The patient denied diplopia or other visual disturbances but did present with bilateral subconjunctival hemorrhage lasting approximately one week.

Angiographic and MRI imaging revealed no aneurysmal dilations, dissection flaps, or arteriovenous malformations. Laboratory tests were within normal limits, except for a mild thrombocytopenia.

Conclusions and discussion. The etiologies of meningeal syndrome include infectious causes, toxins, SAH, tumors, trauma, and electrolyte imbalances. In this case, the definitive diagnosis was based on the clinical presentation, laboratory tests without abnormalities suggestive of other causes, and imaging investigations with findings specific to SAH.

Keywords: subarachnoid hemorrhage, thunderclap headache, idiopathic SAH, non-aneurysmal, meningeal irritation

P134. A brain turned to stone: an unusual manifestation of hypoparathyroidism

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Primary hypoparathyroidism is a rare condition caused by the absence or deficiency of PTH, leading to hypocalcemia. The presentation of idiopathic hypoparathyroidism with classic features of parkinsonism, seizures and cognitive impairment with extensive brain calcification is uncommon. Acquired intracranial calcification that extends beyond the basal ganglia is a rarely observed phenomenon.

A 53-year-old man presented to the emergency department with symptoms suggestive of generalized tonic-clonic seizure. Laboratory examination showed hypocalcemia, hyperphosphatemia and low PTH, indicating hypoparathyroidism. A native brain CT scan revealed dense calcium deposits in the corona radiata, basal ganglia, posterior thalamus, hippocampus, occipital cortex, cerebellar white matter and central pons regions. Throughout the course of the patient's evolution, the condition worsened; extrapyramidal signs such as tremor, rigidity, bradykinesia, hypomimic facies, parkinsonian gait and bilateral presence of the Noica sign were noted. It is worth mentioning that these signs could be influenced by the presence of ankylosing spondylitis. In addition, he experienced cognitive impairment. The patient received seizure management, anti-parkinsonian therapy and maintenance medication of calcium with vitamin D. Regarding the seizures, no signs of relapse were observed. As for the other symptoms, they exacerbated, reflecting the progression of cerebral impairment.

This case describes the rare finding of multiple intracranial calcifications in a case of idiopathic hypoparathyroidism with its various neurological manifestations. When a significant number of calcifications is present, the treatment response may be altered. Therefore, it is imperative to highlight the importance of early presentation to a healthcare provider at the onset of symptoms.

Keywords: intracranial calcification, parkinsonian features, seizures, hypoparathyroidism

P135. Herpes encephalitis: a case of stroke mimic

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Introduction. Encephalitis caused by herpes simplex virus type 1 stands out as one of the primary causes of viral encephalitis worldwide, accounting for about 90% of cases. It is associated with focal or global dysfunction; the most frequent neurological symptoms are confusion, seizures, speech difficulties, paralysis, and coma.

Case presentation. We are presenting the case of a 69-year-old man with a previous history of liver cirrhosis who presented with an acute onset, with fever (39 °C), left side hemiparesis, and speech disorders. The patient also had two seizures on the day of onset. On arrival, his GCS was 15; the neurological assessment showed left facial droop, dysphagia, decreased strength (3/5) in the left upper and lower limbs, and aphasia. The head CT scan didn't reveal ischemic lesions. The brain MRI described an extensive right temporo-insular lesion with hyperintensity on T2 and FLAIR, with diffusion restriction, suggestive of an ischemic stroke in the territory of the right middle cerebral artery. He was admitted to the neurological intensive care unit. He was started on empiric intravenous acyclovir for concern for HSVE. The cerebrospinal fluid study showed 150 WBC, 29 glucose, 462 protein, and a PCR test positive for HSV-1. Afterwards, he was transferred to the infectious diseases clinic for specialized treatment.

Conclusion. This case highlights the importance of performing a differential diagnosis between encephalitis caused by herpes simplex virus and acute ischemic stroke, as well as the necessity of promptly initiating antiviral treatment in suspected cases of HSVE to improve outcomes.

Keywords: herpes encephalitis, HSVE, stroke

P136. Prognostic factors in a case of acute demyelinating polyradiculoneuropathy with severe progression

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Introduction. Acute polyradiculoneuropathy (Guillain-Barré syndrome) is a rare, immune-mediated, self-limiting peripheral neuropathy, with an annual prevalence of 1–2 per 100,000. It presents with symmetrical clinical manifestations (motor and sensory deficits, often accompanied by autonomic symptoms). The prognosis is variable, with both clinical and electrophysiological factors involved.

Methods and results. We present the case of a 43-year-old woman, a kindergarten teacher, diagnosed in our clinic with acute polyradiculoneuropathy—based on clinical and paraclinical findings: nerve conduction studies and cerebrospinal fluid analysis.

From the patient's recent history, we note an influenza vaccination 10 days prior to admission. Two days after symptom onset: tetraparesis with predominant paraparesis, globally absent osteotendinous reflexes, reported tetralimb paresthesia (more pronounced in the lower limbs than the upper), and deep sensory impairment in both lower limbs. Rapid deterioration followed, and two days after admission, the patient developed flaccid tetraplegia (0/5 on the BMRC scale), along with respiratory and hemodynamic decline. She required continuous care in the intensive care unit for 3 months, after which she was transferred to the rehabilitation service.

Five sessions of plasma exchange were performed, followed by the administration of intravenous human immunoglobulins (IVIg) at a dose of 0.4 g/kg body weight/day for five consecutive days.

Electrophysiological studies revealed significant demyelination as well as severe axonal damage, evidenced by markedly prolonged latencies and absent motor responses in nerve conduction studies. Anti-ganglioside antibodies were negative.

Conclusion and discussion. Acute polyradiculoneuropathy is a condition that can follow a severe, fulminant course, accompanied by numerous complications, as illustrated in the case presented. In this patient, the onset of Guillain-Barré syndrome was associated with the administration of the influenza vaccine.

Prognostic assessment in such cases is based on clinical and paraclinical elements, as outlined in recent guidelines.

Keywords: acute polyradiculoneuropathy, neurography, ICU management, prognostic factors

P137. Triple seronegative myasthenia with vegetative manifestations and severe evolution – case report

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Triple seronegative myasthenia accounts for approximately 4% to 15% of all patients diagnosed with this condition. Case series (Morena 2021, Rodiguez et al. 2024, Li 2023) suggest that most patients present with mild clinical signs at diagnosis and have a favourable evolution.

We present the case of a 42-year-old woman with no known pathological conditions at the time of onset of symptoms, who experienced a rapid and severe disease progression. Symptoms began with phonation disturbances, followed by dysphagia, generalized muscle weakness, vision disturbances, and respiratory dysfunction. Over time, she developed tachycardia, excessive sweating, and orthostatic hypotension.

Diagnostic workup.Test results were negative for acetylcholine receptor antibodies (repeated over time), anti-MuSK, anti-LRP4, P/Q-type and N-type calcium channel antibodies, neuronal antigen IgG panel, and anti-ryanodine receptor antibodies.

Imaging studies included brain CT, brain MRI, repeated thoracic CT scans, CT scans of the thorax-abdomen-pelvis with contrast, and cervical region CT — all without significant findings. Electromyography (EMG) revealed a decrement pattern characteristic of myasthenia gravis.

Treatment and evolution. The patient responded to high doses of pyridostigmine (450–600 mg/day) and is under treatment with mycophenolate mofetil (1000 mg/day) and methylprednisolone (alternate regimen of 16 mg and 8 mg, as higher doses were not tolerated). The disease course has been severe, with multiple exacerbations showing poor response to plasmapheresis (two treatment courses — first with five sessions, second with seven sessions) but a favourable response to intravenous immunoglobulin therapy, from the onset of symptoms (June 2024) and the first decompensation (September 2024) until now, requiring 2 courses of plasmapheresis and 6 courses of i.v. immunoglobulins.

Even in stable periods, the patient consistently exhibits dysphonia, dysphagia (requiring gastrostomy feeding in the latter part of the day), mild generalized weakness, exertional dyspnea, and requires nighttime oxygen therapy. Currently, she requires the administration of IV immunoglobulins every 4 weeks, with worsening of symptoms after the second week following the end of each cure.

Conclusions.The absence of autoantibodies often delays diagnosis and appropriate treatment, necessitating correlation with neurophysiological assessments such as repetitive nerve stimulation (RNS) and single-fibre electromyography (SFEMG).

Recognizing symptoms suggestive of a myasthenic syndrome and coordinating clinical and paraclinical investigations is crucial for conducting a complex differential diagnosis to reach a definitive conclusion.

We mention that the RIA method was used to test the antibody against acetylcholine receptors, but in recent years the literature describes (Vinciguerra et al. 2023, Sorrenti et al. 2024) a new technique CBA (live cell-based assay) with greater specificity.

The literature (Lorenzoni et al. 2012, Kao et al. 2018, Harper 2004) describes severe-onset congenital seronegative myasthenia gravis cases with adult-onset symptoms. Specific tests have been collected, and results are pending during manuscript preparation.

In the literature (Sorrenti et al. 2024) there is a described case with triple seronegative generalized myasthenia gravis, refractory to treatment, in which treatment with an FcRn inhibitor (Efgartigimod) was administered and the evolution was favorable, with a significant improvement in MG-ADL and QMG scores.

P138. Neurocardiogenic syndromes after stroke: A case of takotsubo cardiomyopathy revealing the brain-heart axis

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The brain-heart axis plays a crucial role in post-stroke cardiovascular complications, collectively referred to as stroke-heart syndrome (SHS). Central auto-

nomic dysregulation, heightened sympathetic outflow, and catecholaminergic toxicity are key elements in its pathophysiology. Stroke heart syndrome encompasses a spectrum of cardiac manifestations following acute cerebrovascular events – particularly ischemic stroke and subarachnoid hemorrhage – including cardiac arrhythmias, heart failure, and coronary syndromes. Other related conditions include sudden cardiac death and takotsubo syndrome.

We present the case of a 61-year-old woman with a right insular stroke who developed chest pain and dyspnea within 72 hours. She exhibited elevated CK-MB levels and ST-segment elevation in the V2–V6 precordial leads. Transthoracic echocardiography showed normal-sized cardiac chambers, severe left ventricular systolic dysfunction (ejection fraction 15–25%), and apical ballooning. Coronary angiography revealed no obstructive coronary artery disease. The patient recovered, and the final diagnosis was takotsubo cardiomyopathy.

This case highlights the need to assess post-stroke cardiac dysautonomia. Cardiac complications significantly worsen prognosis and represent the second leading cause of death in stroke patients. The bidirectional relationship between the brain and heart underscores the importance of integrated neurocardiac approaches in both acute and long-term care. A deeper understanding of these syndromes may shed light on the broader spectrum of autonomic dysfunction-related cardiomyopathies and improve patient outcomes through early recognition and targeted treatment strategies.

P139. Neurological manifestations of hyperkalemia

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Introduction. Hyperkalemia is a common and potentially life-threatening electrolyte disorder, frequently encountered in the context of renal failure. While cardiac alterations are the most well-documented complications, hyperkalemia can also affect the nervous and muscular systems, leading in extreme cases to flaccid paralysis. Early recognition of this association is therefore essential to prevent severe complications.

Case presentation. We present the case of an 81-year-old patient with a history of hypertension, diabetes, benign prostatic hyperplasia, and chronic kidney disease, under treatment with antiplatelet, peripheral vasodilator, and lipid-lowering agents. The patient presented with acute bilateral lower limb motor deficit (flaccid paraparesis) that developed over 24 hours. Extensive investigations, including laboratory tests, neuroimaging exploration, cerebrospinal fluid analysis, revealed significant hyperkalemia (K=7.9 mEq/L). As no other cause could explain the neurological symptoms, treatment was directed toward correcting the electrolyte imbalance, resulting in initial improvement of paraparesis. However, 10 days later, the patient suddenly developed left-sided motor deficit and facial asymmetry, in the context of an ischemic stroke confirmed by cranial CT. This raises the question of whether hyperkalemia could be a contributing factor in the occurrence of cerebrovascular events, warranting further research.

Conclusion. Hyperkalemia remains a significant electrolyte disorder associated with various neurological manifestations. Its therapeutic management requires an interdisciplinary approach and careful monitoring.

Keywords: hyperkalemia, flaccid paralysis, stroke

CASE SERIES

P156. Spinocerebellar ataxia type 1 (SCA1) – the phenotypic analysis within an affected family

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Introduction. Spinocerebellar ataxia type 1 (SCA1) is an autosomal-dominant hereditary disease. Its main clinical characteristics are represented by progressive cerebellar ataxia, dysarthria, and bulbar dysfunction. The cause is an abnormal CAG repeat expansion identified in the ATXN1 gene.

Methodology. We included 4 out of 5 symptomatic family members, for whom the diagnosis of SCA1 had been established: the index patient and the patient's sister, aunt, and uncle; the patient's mother, although symptomatic, could not be included due to severe disability. The medical history of the individuals and their medical documents were assessed.

Results. The mean age of onset was 46.5 years, with a mean disease duration of 8.25 years. The dominant clinical traits were truncal ataxia, limb ataxia, and gait impairment, though non-ataxia signs were also present. Dedicated scales for rating ataxia were used to determine the degree of disability in the subjects. Cerebral MRI revealed cerebral and cerebellar atrophy. Genetic testing for spinocerebellar ataxias was positive for SCA1 in all tested patients. Clinical variability was observed regarding the age of onset and spectrum of clinical manifestations despite a similar number of repeats in the tested patients.

Conclusions. This phenotypic analysis highlights the clinical variability and difficulty in predicting disease evolution and degree of progression based on the number of CAG repeats, with many risk-modifying factors still unresolved. Genetic testing proves important for a definitive diagnosis and future treatment methods.

Keywords: hereditary ataxia, spinocerebellar ataxia, SCA1

P157. Treatment of neurological autoimmune diseases with plasmapheresis – Our center's experience 2020–2024

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Plasmapheresis is an extracorporeal method for separating the cellular components of blood from plasma, which contains autoantibodies, inflammatory cytokines, circulating immune complexes, toxins, or drugs. The removed plasma is replaced with donor plasma or colloidal solutions.

Materials and methods. We conducted a retrospective clinical study including 52 patients hospitalized in our clinic between 2020 and 2024, who underwent 303 plasmapheresis sessions using the Spectra Optia system. The monitored parameters included demographic characteristics, anthropometric indicators, clinical, and biological data.

Results. A comparison of laboratory tests before the first plasmapheresis session and after the final treatment session showed a significant decrease in hemoglobinin 59.6% of cases (ranging from 0.07 to 3.7 g/dL, $p < 0.05$) and hematocrit in 61.4% of cases (ranging from 0.2 to 13.2%, $p = 0.0008$), without reaching the threshold for severe anemia. Similarly, platelets showed a decreasing trend in 56.8% of cases, though without statistical significance, while fibrinogen decreased in 71.4% of cases ($p = 0.0001$), emphasizing the need for hemostatic profile monitoring. The biochemical profile indicated a tendency for serum protein reduction without significantly affecting albumin levels and without reaching severe hypoproteinemia. Plasmapheresis was generally tolerated, with most sessions proceeding without incidents. When adverse events occurred, they were predominantly mild (vascular access issues, minor allergic reactions) and resolved promptly with treatment.

Conclusions. The effectiveness and favorable tolerance profile reinforce plasmapheresis as a valuable therapeutic option in the management of neurological autoimmune diseases, provided that patients are carefully selected and that potential adverse effects are rigorously monitored and managed.

Keywords: plasmapheresis, efficacy, tolerance

P158. Etiology, evolution, and prognosis of cervico-cerebral arterial dissections – a one-year case series (2024) at the Neurology Clinic in Sibiu

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Introduction. This study retrospectively analyzes the potential risk factors and the clinical-evolutionary characteristics of patients with ischemic stroke secondary to cervico-cerebral arterial dissections.

Material and method. The study included 11 patients admitted to the Neurology Clinic in Sibiu with a diagnosis of cervico-cerebral arterial dissection and ischemic stroke over one calendar year. Clinical, ima-

ging, and therapeutic data were retrospectively analyzed.

Results. The mean age was 46.27 years, with a male predominance (81.82%). Frequent risk factors included smoking (54.54%), occasional alcohol consumption (81.81%), hypertension (36.36%), and dyslipidemia (27.27%). Only one patient had a history of repetitive cervical trauma. Dissections were unilateral, predominantly affecting the internal carotid artery (81.81%). The majority of patients presented with arterial occlusion (72%) confirmed angiographically. The mean NIHSS score at admission was 11 points, improving to 7 points at discharge. The average time from symptom onset to diagnosis was 7 hours. Revascularization therapy included thrombectomy (36.36%) or thrombolysis combined with thrombectomy (27.27%). Among patients treated endovascularly, 85% achieved a TICI 3 score. The mean Rankin score at discharge was 2, and the average hospital stay was 14 days. Complications included hemorrhagic transformation (36.36%), cerebral edema (18.18%), stent reocclusion (9%), and systemic complications (36.36%).

Conclusions. Cervical arterial dissection remains a significant cause of ischemic stroke in young patients, with its etiology still uncertain.

P159. Parkinson's disease or atypical parkinsonism? A challenge in early diagnosis

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Introduction. The diagnosis of movement disorders presents a significant clinical challenge due to the overlapping clinical features shared among various conditions. Disorders such as idiopathic Parkinson's disease, multiple system atrophy, progressive supranuclear palsy, and dementia with Lewy bodies often exhibit similar manifestations, particularly in early stages. The differential diagnosis between neurodegenerative and vascular etiologies adds another layer of complexity to clinical assessment. The common core of these conditions is represented by the presence of bradykinesia, tremor, and rigidity.

Materials and methods. We present a series of four clinical cases. Three patients were initially diagnosed with Parkinson's disease but, due to rapid progression and the appearance of new features, the diagnoses within the neurodegenerative atypical parkinsonism spectrum were revised to: multiple system atrophy – cerebellar form, possible progressive supranuclear palsy, and probable dementia with Lewy bodies. The fourth case was diagnosed with vascular parkinsonism. It is important to mention that definitive diagnosis of neurodegenerative diseases is only possible post-mortem through autopsy, as cerebral cellular changes secondary to abnormal protein accumulation cannot be detected by standard imaging methods, and currently no specific biological markers are available.

Conclusions. Differential diagnosis between Parkinson's disease and atypical parkinsonism remains challenging due to overlapping symptoms and clinical variability. Thorough clinical examination, brain imaging, and monitoring of symptom progression can help improve diagnostic accuracy, aiming for individualized therapeutic interventions and improved patient quality of life.

P160. Vitamin B12 deficiency – clinical, electrophysiological, and imaging correlations – case series

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Background. The most common manifestation of vitamin B12 deficiency is megaloblastic anemia, accompanied by nonspecific symptoms such as asthenia and fatigue. Neurological manifestations may also occur, though specific symptoms often develop only in advanced stages. Vitamin B12 deficiency can cause peripheral neuropathy and, less frequently, degeneration of the lateral and posterior columns of the spinal cord due to demyelination, leading to ataxia and motor deficits. Neuropsychiatric symptoms associated with megaloblastic anemia may be present even without polyneuropathy and are important diagnostic clues. Early diagnosis and vitamin B12 supplementation can prevent irreversible nervous system damage.

Material and methods. We analyzed six patients who presented to our center between 2022 and the

present. All underwent investigations to determine the etiology of their vitamin B12 deficiency.

Results. Clinical neurological features at presentation included gait and balance disturbances, ataxia, muscle weakness, sensory disturbances, and burning dysesthesias in the lower limbs, with subacute or chronic onset. The age range was 26–67 years, with a female-to-male ratio of 2:3. The youngest patient, a male, had type 1 diabetes mellitus and known diabetic polyneuropathy. Two female patients without significant comorbidities showed complete remission of neurological symptoms after vitamin B12 supplementation. MRI in three patients revealed myeloneuropathy with involvement of posterior and lateral columns, presenting as ataxic tetraparesis. Nerve conduction studies in all patients indicated predominantly sensory and motor axonal polyneuropathy of varying severity.

Conclusions. Vitamin B12 deficiency generally responds well to supplementation if diagnosed early, allowing correction of hematological and neurological dysfunctions. Advanced deficiency may cause irreversible spinal cord lesions with posterior column demyelination and disabling symptoms such as severe sensory ataxia, spastic para- or tetraparesis, advanced peripheral neuropathy, cognitive impairment including dementia or refractory depression, and optic neuritis with permanent visual loss – particularly when comorbidities exacerbate chronic deficiency.

P161. Creutzfeldt-Jakob disease: clinical variability and diagnostic challenges

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Introduction. Creutzfeldt-Jakob disease (CJD) is a rare, fatal prion-induced spongiform encephalopathy characterized by rapidly progressive neurodegeneration. The clinical onset can vary significantly between cases, often causing delays in early diagnosis. We present three cases illustrating the clinical variability and diagnostic challenges of CJD.

Case presentations.

Case 1: A 50-year-old male with no known comorbidities presented with muscle weakness, sensory disturbances on the left hemibody, and dysarthria, initially suggesting an acute ischemic event. One month later, behavioral disturbances with affective lability, mixed insomnia, psychomotor agitation, and complex partial seizures developed.

Case 2: A 38-year-old male with a history of kidney transplantation presented with painless visual acuity loss in the right eye, initially suspected as inflammatory optic neuropathy. The left eye became affected subsequently. One month later, altered consciousness progressed to coma.

Case 3: A 67-year-old male with diabetes and multiple cardiovascular conditions presented with altered consciousness, temporal-spatial disorientation, and confusion.

Conclusion. The variability in disease onset underscores the importance of maintaining a high degree of clinical suspicion and pursuing thorough paraclinical investigations for accurate diagnosis. Early recognition is crucial to optimizing diagnosis and patient care.

Keywords: Creutzfeldt-Jakob disease, prion, motor deficit, confusion, visual acuity

P162. Fatigue in Parkinson's disease: prevalence and risk factors

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Background. Fatigue is one of the most frequently encountered non-motor symptoms in Parkinson's disease (PD), significantly impacting all aspects of daily life.

Objective. To determine the prevalence of fatigue among people with PD compared to the general population and to identify factors associated with fatigue.

Methods. In this case-control study, 131 PD patients and 131 healthy controls were enrolled. Fatigue in PD was assessed using the Fatigue Impact Scale (FIS), Chalder Fatigue Scale, and Parkinson Fatigue Scale (PFS-16).

Results. Fatigue was more frequent in PD patients compared to healthy controls. PD patients scored higher on all items of the Chalder scale, with items 1–9 showing statistically significant differences ($p < 0.05$). On the FIS, PFS-16, and Chalder scales, progressively higher scores were found in patients with advanced Hoehn and Yahr stages compared to those with early PD ($p < 0.001$ for FIS and PFS-16; $p = 0.10$ for Chalder). Patients with fatigue reported fewer estimated hours of sleep per night compared to those without fatigue (6.04 ± 1.76 hours vs. 6.90 ± 1.41 hours, $p = 0.002$). Disease duration, motor status, presence of sleep di-

sorders, insomnia, other non-motor symptoms, pain, and excessive daytime sleepiness (EDS) were all independently associated with fatigue ($p < 0.001$).

Conclusion. Fatigue prevalence is higher in PD and increases with disease progression. Fatigue is also associated with a wide range of other non-motor symptoms.

Keywords: Parkinson's disease, fatigue, assessment

P163. Nocturia and sleep disturbances in Parkinson's disease

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Background. One of the most common issues reported by patients with Parkinson's disease (PD) is nocturia, which results in disrupted sleep and frequent awakenings.

Objective. To determine the impact of nocturia on the quality of sleep in PD patients.

Method. A total of 131 patients with Parkinson's disease were enrolled in this observational study. Urinary dysfunction was identified using the Non-Motor Symptoms Questionnaire (NMSQ), and sleep quality was assessed by the Pittsburgh Sleep Quality Index (PSQI), Scales for Outcomes in Parkinson's Disease (SCOPA) sleep components – covering nocturnal and daytime symptoms – and the REM Sleep Behavior Disorder Screening Questionnaire (RBDSQ).

Results. Based on responses to question 9 of the NMSQ ("Getting up regularly at night to pass urine"), patients were divided into two groups: nocturia present (113, 86.25%) and nocturia absent (18, 13.74%). Patients with PD and nocturia showed significantly higher scores on all sleep scales compared to those without nocturia (PSQI, $p = 0.003$; SCOPA nocturnal component, $p = 0.001$; SCOPA daytime component, $p = 0.005$; RBDSQ, $p = 0.002$). Moreover, strong positive correlations were identified between nocturia and all seven components of the PSQI scale ($r > 0.8$), indicating a close association with sleep disturbances.

Conclusion. Patients with Parkinson's disease who experience nocturia have poorer sleep quality than those without urinary problems, due to frequent sleep disruptions. This significantly increases their overall burden.

Keywords: Parkinson's disease, nocturia, sleep disturbances

P164. Prevalence of insomnia in Parkinson's disease and its characteristics

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Background. Among the sleep disorders associated with Parkinson's disease (PD), insomnia plays an important role.

Objective. To determine the prevalence of insomnia among PD patients and to identify its associated features.

Method. In this case-control study, two groups were enrolled: 131 patients with PD and 131 healthy controls. Insomnia was assessed and characterized using the Athens Insomnia Scale (AIS) and the Insomnia Severity Index (ISI).

Results. More than half (54.96%) of the PD patients had insomnia based on ISI scores ≥10, compared to 27.48% in the control group. Using the AIS (cutoff ≥6), insomnia was identified in 86 PD patients (65.64%) and in 40 individuals from the control group (30.53%) ($p < 0.001$).

P165. Rare cases of non-traumatic paraplegia

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Background. Paraplegia is characterized by motor deficits secondary to lesions of the pyramidal tracts. Diagnosis typically relies on identifying acquired causes, while hereditary causes can be difficult to pinpoint.

Case presentations.

Case 1: A 54-year-old female with progressive lower limb motor deficits and urinary incontinence for ten years. Genetic testing revealed an AP5Z1 mutation, confirming hereditary spastic paraplegia.

Case 2: A 52-year-old male presenting with paresthesia and progressive lower limb weakness accompanied by sphincter incontinence for one year. Neurological examination showed sensory loss below the D10 dermatome. Thoracic spine MRI revealed a fusiform

intramedullary lesion; histopathology confirmed dorsal ependymoma.

Case 3: A 68-year-old male with sudden-onset progressive lower limb weakness over one week. Examination showed hypoesthesia to exteroceptive, thermal, and pain sensations, with a sensory level at D6. Thoracic spine MRI identified transverse myelitis. Biological tests confirmed Coxsackie B virus infection.

Case 4: A 57-year-old male reported sudden back pain radiating to lower limbs and motor deficits. MRI revealed a dorsal spinal infarction secondary to an arteriovenous fistula.

Conclusions. Diagnosing paraplegia remains challenging, and although rare, non-traumatic etiologies such as hereditary spastic paraplegia, tumors, infections, and vascular lesions should be considered.

Keywords: paraplegia, non-traumatic, hereditary spastic paraplegia, ependymoma, transverse myelitis, arteriovenous fistula

P166. Diagnosing Creutzfeldt-Jakob disease: a closer look at early symptoms

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Introduction. Creutzfeldt-Jakob disease (CJD) is a rare and rapidly progressive neurodegenerative disorder. It typically presents with dementia and neuropsychiatric features such as myoclonus, cerebellar dysfunction, extrapyramidal signs, and eventually akinetic mutism. Early symptoms may mimic other dementias, but the younger age of onset and swift progression are characteristic of CJD.

Methods. We retrospectively reviewed the hospital records of patients discharged with a diagnosis of CJD over the past ten years. Clinical presentation, MRI findings, and diagnostic modalities were analyzed for seven confirmed cases.

Results. Six of the seven patients (five women, one man) presented with rapidly progressive dementia, hallucinations, and neuropsychiatric decline. One patient initially exhibited isolated cerebellar symptoms (ataxia and vertigo).

MRI typically showed FLAIR hyperintensities in the basal ganglia and cortical regions, especially in the parietal, occipital, and frontal lobes. The most frequent pattern was combined cortical and striatal involvement.

Cerebrospinal fluid (CSF) testing was performed in all cases: six were positive for 14-3-3 protein, while one was diagnosed via RT-QuIC. While the 14-3-3 test shows ~92% sensitivity and 95% specificity in classic cases, specificity decreases in atypical presentations. RT-QuIC, with over 80% sensitivity and near 100% specificity, is now the preferred confirmatory method.

Conclusion. CJD should be considered in younger patients with rapidly progressive cognitive decline. When clinical suspicion is high, RT-QuIC testing of CSF plays a crucial role in establishing a definitive diagnosis.

Keywords: Creutzfeldt-Jakob disease, prion disease, rapidly progressive dementia, MRI, RT-QuIC, 14-3-3 protein

P167. Autoimmune encephalitis in patients with systemic lupus erythematosus: a systematic review of the literature

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Introduction. Recognizing the overlap of autoimmune disorders is increasingly valuable as targeted therapies become more widely available. Neuropsychiatric systemic lupus erythematosus (NPSLE) remains a challenging and often ambiguous diagnosis, relying heavily on clinical judgment. Identifying associated conditions such as autoimmune encephalitis may improve our understanding of neurological manifestations in lupus patients and guide more appropriate treatment.

Methods. A systematic review of the literature was conducted to identify reported cases of autoimmune encephalitis in patients with systemic lupus erythematosus (SLE). Other immune-mediated encephalitis syndromes were included to broaden the evaluation of brain inflammation in this patient population.

Results. We identified full-text articles describing 8 seropositive autoimmune encephalitis cases, 7 limbic encephalitis cases, 10 striatal encephalitis cases, and 8 cases of other immune-mediated encephalitides. Clinical features and treatment responses were summarized for each category.

Conclusions. Autoimmune encephalitis most often occurred after a prior diagnosis of lupus, with or wi-

thout evidence of active disease. First-line immunotherapy led to complete remission in most patients; cyclophosphamide was used in select cases. We recommend considering overlap syndromes when autoimmune encephalitis criteria are met, especially in the presence of inflammatory neuroimaging, specific clinical features, or electroencephalographic abnormalities.

Keywords: systemic lupus erythematosus, autoimmune encephalitis, limbic encephalitis, neuropsychiatric lupus, overlap syndrome

P168. Evaluating the role of ASL-MRI as an alternative to FDG-PET in neurodegenerative disease diagnosis

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Introduction. FDG-PET is widely used in the diagnosis of neurodegenerative diseases but is costly and not universally available. Arterial spin labeling MRI (ASL-MRI) is a non-invasive, lower-cost imaging technique that may serve as a potential alternative.

Methods. We analyzed six patients with various neurodegenerative diseases: two with frontotemporal degeneration (FTD), two with Lewy body dementia, and two with corticobasal degeneration. In each group, one patient underwent ASL-MRI and the other FDG-PET. All patients showed parkinsonism unresponsive to Levodopa.

Results. In FTD, ASL-MRI revealed frontotemporal hypoperfusion corresponding to metabolic deficits identified on FDG-PET. In Lewy body dementia, both methods showed bilateral parieto-occipital hypoperfusion with hyperperfusion in the cingulate cortex. Corticobasal degeneration cases demonstrated asymmetric frontoparietal hypoperfusion and metabolic decline.

Conclusion. ASL-MRI yields comparable diagnostic information to FDG-PET in neurodegenerative diseases and represents a cost-effective, more accessible alternative, particularly useful in resource-limited settings.

Keywords: ASL-MRI, FDG-PET, neurodegenerative disease, frontotemporal dementia, Lewy body dementia, corticobasal degeneration

P169. Evaluating the role of ASL-MRI as an alternative to FDG-PET in neurodegenerative disease diagnosis

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Objective. To retrospectively evaluate clinical characteristics, treatment strategies, and outcomes in patients with multiple sclerosis (MS)-associated optic neuritis (ON), with a focus on visual and optical coherence tomography (OCT) findings in corticosteroid-only versus adjunctive plasma exchange (PLEX) therapy.

Methods. The records of 15 ON patients (diagnosed: 2023 – 2024) were reviewed. Demographic, clinical, and treatment data – including time to diagnosis and treatment initiation – were extracted. Visual acuity was measured in logMAR, and OCT parameters (mean retinal nerve fiber layer [RNFL] and ganglion cell layer [GCL] thickness) were obtained at baseline and follow-up.

Results. The cohort was predominantly young females (93%, median age 35). Median time to diagnosis was 5 days. All patients received high-dose corticosteroids. 33% required PLEX due to an unsatisfactory steroid response. In the corticosteroid-only group, the mean baseline logMAR was 0.75, improving to 0.15 (near-normal vision) at follow-up, with minimal RNFL (–2 µm) and moderate GCL thinning (–8.75 µm). The PLEX group had a poorer baseline (mean logMAR 1.26) that improved to 0.26, yet exhibited marked structural damage (RNFL thinning:–22.5 µm, GCL thinning: –20.8 µm).

Conclusion. These findings indicate that although both treatments yield visual improvement, patients requiring PLEX exhibit greater initial impairment and pronounced retinal thinning. Our PLEX utilization rate (33%) exceeds the current ~20% typically reported in MS-associated ON, and the median PLEX initiation time (23 days) is slightly delayed relative to the optimal 14–21-day window. Notably, all patients were diagnosed with MS, contrary to literature linking severe ON primarily to NMOSD or MOGAD. Early identification and timely treatment escalation may improve outcomes and warrant refined management protocols.